

MEDICAL ENGLISH CURRICULUM

Competency-Based Programme for General Doctors

English Proficiency Levels: B1 – B2 (CEFR)

Programme Length: 40 Sessions

Session Duration: 60 Minutes

Target Learners: General Medical Practitioners

For Use By: English Language Teachers

Designed & Authored by **Rayene Medabis**

English Educator & Curriculum Developer

This curriculum and all instructional materials contained herein are the original intellectual property of Rayene Medabis. All rights reserved.

PART ONE: CURRICULUM OVERVIEW

1. Introduction

This curriculum is a 40-session, competency-based Medical English programme designed for general medical doctors operating at B1 and B2 levels of English proficiency on the Common European Framework of Reference for Languages (CEFR). It has been developed for delivery by English language teachers in medical education settings, with no prerequisite specialisation in medicine required of the teacher.

The curriculum follows a progressive structure, building from foundational medical vocabulary and professional identity through to complex, sensitive clinical communication skills. Each session integrates linguistic competencies with authentic medical communication tasks, ensuring that language learning is always embedded in real clinical purpose.

2. Target Learners

Primary audience: General medical doctors and physicians

English level: B1 (intermediate) to B2 (upper-intermediate) on the CEFR

Context: Doctors working in or preparing to work in English-speaking clinical environments

Prior knowledge: Full medical training; no specific English for Medical Purposes background required

3. Programme Goals

The overarching goals of this curriculum are to enable doctors to:

- Communicate confidently and accurately with patients, families, and colleagues in English across a full range of clinical contexts.
- Develop language competence specifically suited to the professional demands of medical practice in English-speaking environments.
- Produce clear, correct, and contextually appropriate spoken and written medical communication.
- Build professional confidence, reducing the anxiety that often accompanies clinical communication in a second language.
- Develop lifelong medical English learning strategies and metacognitive awareness.

4. Programme Objectives

By the end of this 40-session programme, learners will be able to:

1. Conduct a complete patient consultation in English, including history taking, physical examination guidance, diagnosis communication, and treatment planning.
2. Produce accurate, well-structured clinical documentation including SOAP notes, referral letters, discharge summaries, and clinical summaries.
3. Communicate effectively with colleagues in verbal handovers, case presentations, MDT meetings, and clinical briefings.
4. Navigate sensitive and complex clinical communication scenarios including breaking bad news, obtaining informed consent, and managing patient distress.

5. Read and discuss medical literature in English, applying evidence-based language to clinical discussions.
6. Adapt communication style for different audiences (patients, colleagues, superiors), different settings (telehealth, emergency, outpatient), and different cultural backgrounds.
7. Use appropriate registers, with accurate vocabulary, grammar, and discourse conventions across spoken and written medical English.

5. Programme Outcomes

Upon successful completion of the programme, learners will demonstrate:

- **CLINICAL COMMUNICATION COMPETENCE:** Ability to conduct a full patient consultation in English across all phases, from opening to closing, including history taking, examination guidance, investigation ordering, diagnosis delivery, and treatment discussion.
- **DOCUMENTATION COMPETENCE:** Ability to write clear, accurate, and appropriately formatted clinical documents including SOAP notes, referral letters, discharge summaries, and case reports.
- **PROFESSIONAL COMMUNICATION COMPETENCE:** Ability to participate effectively in interprofessional communication including handovers, case presentations, MDT meetings, and collegial discussions.
- **SENSITIVE COMMUNICATION COMPETENCE:** Ability to manage emotionally complex and ethically challenging consultations including breaking bad news, consent processes, cultural differences, and professional boundaries.
- **LANGUAGE SYSTEM AWARENESS:** Improved accuracy in medical English grammar, vocabulary, pronunciation, and written conventions at a B2 level.

6. CEFR Progression Map

Sessions are designed to progress from B1 to B2 across the course of the programme:

Sessions	Level	Focus
1–10	B1	Foundations, basic consultation skills, vocabulary
11–20	B1–B2	Complete history, examination, diagnosis language
21–30	B2	Treatment, documentation, and clinical writing
31–40	B2	Advanced professional, interpersonal, and ethics communication

7. Unit Structure Overview

UNIT 1: MEDICAL FOUNDATIONS: Sessions 1–5

Session 1: Introduction to Medical English & Professional Identity

Session 2: The Human Body – Anatomy Vocabulary in Context

Session 3: Medical Specialties, Roles & Healthcare Settings

Session 4: Numbers, Measurements & Medical Data

Session 5: Professional Register – Formal vs. Informal Language in Medicine

UNIT 2: OPENING THE CONSULTATION: Sessions 6–10

Session 6: Opening Consultations & Establishing Rapport

- Session 7:** Eliciting the Chief Complaint
- Session 8:** Describing and Asking About Symptoms
- Session 9:** Exploring Pain – The SOCRATES Framework
- Session 10:** Active Listening & Empathic Language

UNIT 3: TAKING A COMPLETE HISTORY: Sessions 11–15

- Session 11:** Past Medical History, Medications & Allergies
- Session 12:** Family History & Genetic Risk Communication
- Session 13:** Social History & Lifestyle Factors
- Session 14:** Review of Systems in English
- Session 15:** Pediatric & Geriatric Communication Adaptations

UNIT 4: CLINICAL EXAMINATION & DIAGNOSIS: Sessions 16–20

- Session 16:** The Physical Examination – Instructions & Vocabulary
- Session 17:** Vital Signs – Measurement, Communication & Documentation
- Session 18:** Ordering and Explaining Diagnostic Tests
- Session 19:** Interpreting and Presenting Lab Results & Imaging
- Session 20:** Forming and Communicating a Diagnosis

UNIT 5: TREATMENT & MANAGEMENT: Sessions 21–25

- Session 21:** Treatment Plans – Explaining Options to Patients
- Session 22:** Pharmacology English – Prescriptions, Dosages & Side Effects
- Session 23:** Explaining Procedures and Obtaining Consent
- Session 24:** Post-Treatment Instructions & Follow-Up Care
- Session 25:** Emergency Communication – SBAR and Rapid Handover

UNIT 6: CLINICAL DOCUMENTATION & WRITING: Sessions 26–30

- Session 26:** Writing SOAP Notes in English
- Session 27:** Clinical Summaries and Case Reports
- Session 28:** Referral Letters and Medical Correspondence
- Session 29:** Discharge Summaries
- Session 30:** Medical Terminology – Prefixes, Suffixes & Word Building

UNIT 7: PROFESSIONAL & INTERDISCIPLINARY COMMUNICATION: Sessions 31–35

- Session 31:** Communicating with Colleagues – Handovers & Briefings
- Session 32:** Case Presentations in English
- Session 33:** Multidisciplinary Team (MDT) Meetings
- Session 34:** Telehealth & Remote Patient Communication
- Session 35:** Reading & Discussing Medical Literature

UNIT 8: COMPLEX & SENSITIVE COMMUNICATION: Sessions 36–40

- Session 36:** Breaking Bad News – The SPIKES Protocol
- Session 37:** Informed Consent, Patient Rights & Shared Decision-Making
- Session 38:** Cross-Cultural Communication in Medicine
- Session 39:** Ethics, Confidentiality & Professional Boundaries in English
- Session 40:** Integrated Review, Reflection & Mock Consultation Assessment

PART TWO: SESSION CURRICULUM

This section presents the curriculum for each of the 40 sessions, including CEFR level, competency focus, objectives, outcomes, and key language. These serve as the foundational planning reference for each lesson.

Author: Rayene Medabis

UNIT 1: MEDICAL FOUNDATIONS

Session 1: Introduction to Medical English & Professional Identity

Unit	UNIT 1: MEDICAL FOUNDATIONS
CEFR Level	B1
Competency	Professional self-expression and introductions
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Introduce themselves professionally in English using key career information
- Describe their medical role, specialization, and work context
- Understand the course structure and identify personal language learning goals
- Use basic professional vocabulary related to medicine and healthcare settings

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can deliver a clear, structured 60-second professional self-introduction
- Can describe their medical specialty and daily responsibilities in English
- Can articulate 2–3 specific language goals for the course
- Can participate in a basic peer discussion about professional backgrounds

Key Language & Vocabulary

- Titles & roles: specialist, consultant, resident, general practitioner, attending physician
- Settings: ward, outpatient clinic, emergency department, operating theatre, ICU
- Structures: I work as... / I specialize in... / I am responsible for... / I trained at...
- Questions: What is your specialty? How long have you been practicing? Where do you work?

Session 2: The Human Body – Anatomy Vocabulary in Context

Unit	UNIT 1: MEDICAL FOUNDATIONS
CEFR Level	B1
Competency	Medical vocabulary and anatomical description
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Name major anatomical structures and body systems using correct English terminology
- Use directional anatomical terms (superior, inferior, medial, lateral, proximal, distal)
- Describe the location and function of key organs in English
- Distinguish between medical terminology and lay language for common structures

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can correctly label and describe major anatomical structures
- Can use anatomical directional language in descriptive sentences
- Can explain organ location to a patient using accessible lay language
- Can switch appropriately between medical register and patient-friendly terms

Key Language & Vocabulary

- Systems: cardiovascular, respiratory, gastrointestinal, musculoskeletal, central nervous
- Directional: anterior/posterior, superior/inferior, medial/lateral, proximal/distal
- Structure: The [organ] is located in the [region], just [above/below/beside] the [landmark]
- Lay vs. medical: heart attack / myocardial infarction; kidney stones / nephrolithiasis

Session 3: Medical Specialties, Roles & Healthcare Settings

Unit	UNIT 1: MEDICAL FOUNDATIONS
CEFR Level	B1
Competency	Professional and institutional vocabulary
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Name and describe major medical specialties and their clinical scope
- Identify and describe professional roles within a healthcare team
- Describe different healthcare settings and their functions
- Ask and answer questions about professional roles in English

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can accurately describe their own specialty and those of colleagues
- Can explain the roles of doctors, nurses, allied health, and support staff
- Can describe healthcare settings including hospital departments and primary care
- Can engage in small-talk conversation about professional backgrounds

Key Language & Vocabulary

- Specialties: cardiology, oncology, orthopaedics, neurology, paediatrics, obstetrics
- Roles: attending, registrar, house officer, scrub nurse, physiotherapist, radiographer
- Settings: general ward, HDU, ICU, ambulatory care, surgical unit, outpatient department
- Phrases: She is in charge of... / He works alongside... / The team consists of...

Session 4: Numbers, Measurements & Medical Data

Unit	UNIT 1: MEDICAL FOUNDATIONS
CEFR Level	B1
Competency	Numerical and quantitative communication in medicine
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Say, write, and comprehend numbers, decimals, fractions, and percentages in medical contexts
- Communicate measurements such as height, weight, dosage, and lab values
- Use correct language to describe ranges, trends, and changes in data
- Interpret and report basic clinical data verbally and in writing

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can accurately say and write medical numbers, measurements, and dosages
- Can describe data trends using appropriate language (elevated, decreased, within normal limits)
- Can report a patient's vitals and basic laboratory values clearly
- Can interpret and communicate simple statistical data from a medical context

Key Language & Vocabulary

- Ranges: within normal limits, borderline, mildly/moderately/severely elevated
- Trends: has risen / has fallen / remains stable / is trending downward
- Dosage: take [X] mg twice daily / administer [X] units per hour
- Data framing: The result showed... / The level was... / Compared to baseline...

Session 5: Professional Register – Formal vs. Informal Language in Medicine

Unit	UNIT 1: MEDICAL FOUNDATIONS
CEFR Level	B1–B2
Competency	Register awareness and code-switching in medical communication
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Distinguish between formal, informal, and technical registers in medical English
- Adapt language appropriately for different audiences (patients, colleagues, superiors)
- Identify and avoid jargon when communicating with non-medical audiences
- Use appropriate hedging and softening language in professional contexts

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can identify the appropriate register for a given medical communication context
- Can reformulate technical medical language into accessible patient-friendly explanations
- Can use formal professional language when writing or speaking to superiors
- Can soften directives and requests appropriately (Could you... / It would be helpful if...)

Key Language & Vocabulary

- Formal: I would recommend... / It is advisable that... / The patient presents with...
- Informal: You should... / I think... / The patient has...
- Softening: You might want to consider... / It could be worth discussing...
- Patient-friendly: your heart / your lungs / a high blood sugar level / waterworks

UNIT 2: OPENING THE CONSULTATION

Session 6: Opening Consultations & Establishing Rapport

Unit	UNIT 2: OPENING THE CONSULTATION
CEFR Level	B1
Competency	Patient-centred consultation opening skills
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Use appropriate greetings, introductions, and small-talk to open a consultation
- Establish rapport and build trust through verbal and linguistic strategies
- Demonstrate active interest using listener responses and follow-up questions
- Use open-ended questions to invite the patient to share their reason for visiting

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can open a medical consultation professionally and warmly in English
- Can use small-talk and reassuring language to put patients at ease
- Can use open-ended questions to invite the patient's narrative ('What brings you in today?')
- Can use listener responses (I see / Go on / Tell me more) to encourage the patient

Key Language & Vocabulary

- Greetings: Good morning / Please take a seat / It's nice to meet you / How are you today?
- Opening: What brings you in today? / How can I help you? / What seems to be the problem?
- Listener responses: I see / Go on / Mm-hmm / And then what happened? / Tell me more
- Rapport: I understand this must be difficult / You're in the right place / We'll sort this out

Session 7: Eliciting the Chief Complaint

Unit	UNIT 2: OPENING THE CONSULTATION
CEFR Level	B1
Competency	Structured history taking – presenting complaint
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Use appropriate language to ask about and elicit a patient's chief complaint
- Encourage the patient to describe the problem in their own words
- Identify and use follow-up probing questions
- Clarify and summarise the presenting complaint back to the patient

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can use open and closed questions strategically to explore the chief complaint
- Can invite a patient narrative without interrupting prematurely
- Can use probing follow-up questions to deepen understanding
- Can accurately summarise the chief complaint back to the patient for confirmation

Key Language & Vocabulary

- Elicitation: Tell me about what's been happening / Can you describe it? / When did it start?
- Probing: What happened next? / How has it affected your daily life? / Have you noticed anything else?
- Clarification: So, if I understand correctly... / Let me make sure I have this right...
- Confirmation: Is that right? / Would you say that's accurate? / Have I understood you correctly?

Session 8: Describing and Asking About Symptoms

Unit	UNIT 2: OPENING THE CONSULTATION
CEFR Level	B1
Competency	Symptom elicitation and clinical vocabulary
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Identify and use clinical vocabulary for common symptoms across body systems
- Ask patients to describe symptoms in both lay and medical language
- Describe symptoms using appropriate adjectives (severity, character, quality)
- Distinguish between symptoms reported by patients and signs observed by doctors

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can name and describe common symptoms in both medical and lay language
- Can ask patients to elaborate on symptoms using appropriate descriptive questions
- Can use symptom-describing adjectives correctly (dull, sharp, burning, throbbing)
- Can distinguish between subjective symptoms and objective clinical signs in English

Key Language & Vocabulary

- Symptom types: pain, dyspnoea, nausea, oedema, fatigue, palpitations, dizziness, rash
- Character words: sharp, dull, throbbing, burning, cramping, stabbing, aching, pressure-like
- Severity: mild / moderate / severe / intermittent / constant / worsening / improving
- Questions: How would you describe it? / Is it constant or does it come and go? / On a scale of 1 to 10...

Session 9: Exploring Pain – The SOCRATES Framework

Unit	UNIT 2: OPENING THE CONSULTATION
CEFR Level	B1–B2
Competency	Structured pain assessment and communication
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Apply the SOCRATES framework to conduct a structured pain assessment
- Use a full range of questions to explore each dimension of pain
- Record pain information accurately using clinical language
- Explain pain assessment to a patient in accessible terms

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can systematically explore pain using all 8 SOCRATES dimensions
- Can generate 2–3 questions for each SOCRATES category
- Can document pain findings clearly in a clinical note
- Can explain to a patient why and how their pain is being assessed

Key Language & Vocabulary

- SOCRATES: Site, Onset, Character, Radiation, Associated symptoms, Time, Exacerbating/relieving factors, Severity
- Site: Where exactly is the pain? / Does it stay in one place or move?
- Radiation: Does the pain travel or radiate anywhere?
- Time: Is it constant, or does it come and go? / How long has it been there?

Session 10: Active Listening & Empathic Language

Unit	UNIT 2: OPENING THE CONSULTATION
CEFR Level	B2
Competency	Emotional intelligence and patient-centred communication
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Identify and apply verbal active listening strategies in medical consultations
- Use empathic responses to acknowledge patients' emotions and concerns
- Respond appropriately to unexpected patient disclosures or emotional moments
- Balance clinical efficiency with emotional responsiveness

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can use verbal active listening strategies (reflection, paraphrasing, silence) effectively
- Can identify patient emotions from speech and respond empathically
- Can respond to patient distress using the NURSE framework (Name, Understand, Respect, Support, Explore)
- Can maintain clinical progress while responding to emotional content

Key Language & Vocabulary

- Empathy: I can hear that this has been really difficult for you / That sounds very worrying
- NURSE responses: It sounds like you're feeling... / That must be very hard / I really respect...
- Reflection: You mentioned that... / So you've been feeling... / What I'm hearing is...
- Legitimation: Anyone in your situation would feel that way / It's completely understandable

UNIT 3: TAKING A COMPLETE HISTORY

Session 11: Past Medical History, Medications & Allergies

Unit	UNIT 3: TAKING A COMPLETE HISTORY
CEFR Level	B1–B2
Competency	Systematic history taking – past history
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Ask systematic questions about a patient's past medical and surgical history
- Elicit accurate medication histories including dosages and adherence
- Inquire about allergies and document adverse reactions appropriately
- Use structured PMH/Medication/Allergy language in clinical documentation

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can systematically elicit a past medical and surgical history
- Can document medications with name, dose, frequency, and route
- Can distinguish between allergies and intolerances and ask about both
- Can document PMH, medications, and allergies in standard clinical format

Key Language & Vocabulary

- PMH: Have you ever been diagnosed with...? / Do you have a history of...? / Have you had surgery before?
- Medications: Are you taking any medications? / What dose? / How often? / Do you ever miss doses?
- Allergies: Are you allergic to anything? / What happens when you take it? / How severe was the reaction?
- Documentation format: PMH: HTN, T2DM | Meds: Metformin 500mg BD | Allergies: Penicillin (anaphylaxis)

Session 12: Family History & Genetic Risk Communication

Unit	UNIT 3: TAKING A COMPLETE HISTORY
CEFR Level	B2
Competency	Genetic risk elicitation and patient communication
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Elicit a structured family history relevant to the patient's presenting complaint
- Communicate genetic risk information clearly and sensitively
- Use kinship terminology accurately in English
- Explain hereditary conditions and risk factors in lay language

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can construct a structured family history using appropriate English questions
- Can use correct kinship terms (first-degree relative, maternal, paternal) accurately
- Can explain hereditary risk to a patient using accessible, non-alarming language
- Can document family history clearly in a clinical note or pedigree format

Key Language & Vocabulary

- Questions: Has anyone in your family had...? / On your mother's or father's side? / At what age?
- Kinship terms: first/second-degree relative, maternal/paternal grandparent, sibling
- Risk communication: There is an increased risk... / This does not mean you will definitely...
- Heredity: runs in the family, genetic predisposition, hereditary, family tendency

Session 13: Social History & Lifestyle Factors

Unit	UNIT 3: TAKING A COMPLETE HISTORY
CEFR Level	B2
Competency	Lifestyle and social determinants elicitation
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Elicit a sensitive and comprehensive social history in English
- Ask about smoking, alcohol, recreational drugs, and sexual history appropriately
- Explore occupation, living situation, and social supports
- Use non-judgmental, respectful language when discussing sensitive lifestyle topics

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can ask about all major social history components using appropriate, respectful English
- Can calculate and communicate pack-year history and alcohol units accurately
- Can use non-judgmental framing for sensitive questions (e.g., drug use, sexual history)
- Can integrate social history into a holistic clinical picture

Key Language & Vocabulary

- Smoking: Do you smoke? / How many cigarettes a day? / Pack-year history
- Alcohol: Do you drink alcohol? / How many units per week? / Any binge drinking?
- Non-judgmental framing: Some people find... / I ask everyone this question... / There are no wrong answers
- Social: Who do you live with? / Do you have support at home? / What do you do for work?

Session 14: Review of Systems in English

Unit	UNIT 3: TAKING A COMPLETE HISTORY
CEFR Level	B2
Competency	Systematic ROS elicitation and documentation
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Conduct a systematic review of systems (ROS) in English
- Use appropriate screening questions for each body system
- Distinguish between positive and negative findings in ROS
- Document ROS findings using standard clinical abbreviations and terminology

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can ask at least 2 screening questions per major body system
- Can conduct a complete ROS in under 5 minutes using efficient clinical English
- Can document both positive and pertinent negative findings appropriately
- Can use standard abbreviations correctly (SOB, LOC, CP, N/V, etc.)

Key Language & Vocabulary

- CVS: Any chest pain? / Do you ever feel your heart racing? / Have you had ankle swelling?
- Respiratory: Any shortness of breath? / A cough? / Ever coughed up blood?
- GI: Any change in bowel habits? / Nausea or vomiting? / Weight changes?
- Documentation: (+) SOB on exertion, (-) orthopnoea, (-) PND, (+) mild ankle oedema

Session 15: Pediatric & Geriatric Communication Adaptations

Unit	UNIT 3: TAKING A COMPLETE HISTORY
CEFR Level	B2
Competency	Adapting consultation language for specific patient populations
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Adapt history-taking language for paediatric patients and their parents/carers
- Adjust communication for elderly patients, accounting for cognitive and sensory needs
- Use age-appropriate vocabulary and explanations when speaking with children
- Manage three-way communication between doctor, patient, and carer

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can communicate directly with children using age-appropriate English
- Can include parents/carers in the consultation without excluding the child
- Can adapt pacing, vocabulary, and approach for elderly or cognitively impaired patients
- Can manage the dynamics of a three-way consultation in English

Key Language & Vocabulary

- Paediatric: Can you point to where it hurts? / On a scale of 1 to 10... / Tell me about school
- Carer communication: Could you tell me what you've noticed? / How long has [name] been like this?
- Geriatric: I'd like to take it slowly today / Let me repeat that / Is there anything you'd like to ask?
- Capacity language: Does that make sense? / Can you tell me back what I've just explained?

UNIT 4: CLINICAL EXAMINATION & DIAGNOSIS

Session 16: The Physical Examination – Instructions & Vocabulary

Unit	UNIT 4: CLINICAL EXAMINATION & DIAGNOSIS
CEFR Level	B1–B2
Competency	Giving clear examination instructions and describing findings
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Give clear, polite instructions during a physical examination in English
- Describe physical examination findings using appropriate clinical terminology
- Respond to patient questions or discomfort during the examination
- Use appropriate hedging and permission-seeking before and during examination

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can give a full set of examination instructions clearly and politely
- Can describe examination findings using clinical terms and lay equivalents
- Can address patient concerns or discomfort during examination appropriately
- Can seek permission before each step of a physical examination

Key Language & Vocabulary

- Permission: Is it alright if I examine...? / I'd like to have a feel of your... / This may feel a little cold
- Instructions: Please take a deep breath / Can you roll up your sleeve? / Look straight ahead
- Findings: I can feel a small lump / The area appears to be tender / Breath sounds are reduced on the left
- Comfort: Are you okay? / Let me know if that's uncomfortable / We're nearly done

Session 17: Vital Signs – Measurement, Communication & Documentation

Unit	UNIT 4: CLINICAL EXAMINATION & DIAGNOSIS
CEFR Level	B1–B2
Competency	Vital signs elicitation, interpretation, and reporting
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Correctly communicate all major vital signs in English
- Describe normal and abnormal vital signs using appropriate clinical language
- Interpret vital sign trends and communicate concerns to colleagues
- Document vital signs accurately and in standard clinical format

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can name, measure-describe, and communicate all major vital signs fluently
- Can describe vital sign abnormalities using precise, graded clinical language
- Can report a set of vital signs to a colleague including trends and concerns
- Can document vital signs correctly using standard clinical notation

Key Language & Vocabulary

- Vitals: HR, BP, RR, SpO₂, temperature, GCS, blood glucose, urine output
- Communication: The blood pressure is 150 over 90 / The oxygen saturation is dropping
- Trend language: It's been trending up over the last four hours / It spiked at midnight
- Concern: I'm concerned about... / This is outside normal parameters / We should consider...

Session 18: Ordering and Explaining Diagnostic Tests

Unit	UNIT 4: CLINICAL EXAMINATION & DIAGNOSIS
CEFR Level	B2
Competency	Investigation ordering, explanation, and preparation
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Use appropriate language to order investigations verbally and in writing
- Explain the purpose, procedure, and preparation for common investigations to patients
- Respond to patient questions and concerns about diagnostic tests
- Communicate test results clearly and appropriately to patients and colleagues

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can order common investigations using correct clinical English
- Can explain the purpose and process of a diagnostic test to a patient
- Can answer patient questions about what to expect during a test
- Can communicate whether a test result is normal or abnormal to a patient

Key Language & Vocabulary

- Ordering: I'd like to run some tests / I'm going to request a chest X-ray / We'll send off some blood tests
- Explaining purpose: This test will help us... / We do this to rule out... / It will show us whether...
- Procedure: You'll need to fast for 8 hours / The scan takes about 30 minutes / You'll feel a brief sting
- Results: Everything came back normal / There's one result I'd like to discuss / This is what we call...

Session 19: Interpreting and Presenting Lab Results & Imaging

Unit	UNIT 4: CLINICAL EXAMINATION & DIAGNOSIS
CEFR Level	B2
Competency	Clinical data interpretation and verbal presentation
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Read and interpret common laboratory and imaging results in English
- Describe findings from blood tests, X-rays, CT, and ECGs using clinical language
- Present results clearly and logically to a colleague or in a clinical setting
- Communicate result implications for diagnosis and management

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can read aloud and interpret a standard blood results panel
- Can describe basic X-ray, ECG, and CT findings using clinical terminology
- Can present investigation results in a structured, logical verbal format
- Can communicate the clinical significance of an abnormal result

Key Language & Vocabulary

- Lab language: elevated / reduced / within normal range / borderline / critically low
- ECG: sinus rhythm / rate of / ST elevation / T-wave inversion / left bundle branch block
- CXR: clear lung fields / cardiomegaly / consolidation / pleural effusion / pneumothorax
- Presentation structure: The blood tests show... / Of note... / The most significant finding is...

Session 20: Forming and Communicating a Diagnosis

Unit	UNIT 4: CLINICAL EXAMINATION & DIAGNOSIS
CEFR Level	B2
Competency	Diagnostic reasoning and patient-facing diagnosis communication
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Use clinical reasoning language to formulate a diagnosis in English
- Present a differential diagnosis list with supporting evidence
- Communicate a diagnosis clearly and sensitively to a patient
- Respond to patient questions about their diagnosis

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can formulate and present a differential diagnosis with ranked clinical reasoning
- Can explain a diagnosis to a patient in lay terms, answering follow-up questions
- Can use appropriately hedged language when diagnosis is uncertain
- Can document a working diagnosis clearly in a clinical note

Key Language & Vocabulary

- Differential: My leading diagnosis is... / Other possibilities include... / I want to rule out...
- Certainty levels: I believe... / It is most likely... / We cannot exclude... / This is consistent with...
- Patient explanation: What this means is... / The reason for this is... / In simple terms...
- Uncertainty: We don't have a definitive answer yet / We're still investigating / This takes time to confirm

UNIT 5: TREATMENT & MANAGEMENT

Session 21: Treatment Plans – Explaining Options to Patients

Unit	UNIT 5: TREATMENT & MANAGEMENT
CEFR Level	B2
Competency	Shared decision-making and treatment communication
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Explain treatment options clearly and objectively to a patient in English
- Present the benefits and risks of each treatment option in lay language
- Facilitate shared decision-making by eliciting patient preferences and values
- Summarise an agreed treatment plan clearly

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can present 2–3 treatment options with benefits and risks in accessible language
- Can elicit patient preferences and incorporate them into the treatment discussion
- Can apply the 'Ask-Tell-Ask' shared decision-making technique
- Can summarise and confirm an agreed treatment plan with a patient

Key Language & Vocabulary

- Options: There are a few options we could consider / One approach would be... / Alternatively...
- Benefits & risks: The main advantage is... / There is a risk of... / One potential side effect...
- Eliciting preference: What matters most to you? / How do you feel about...? / What are your thoughts?
- Confirming plan: So we've agreed that... / The plan is to... / Does that feel right to you?

Session 22: Pharmacology English – Prescriptions, Dosages & Side Effects

Unit	UNIT 5: TREATMENT & MANAGEMENT
CEFR Level	B2
Competency	Medication communication with patients and colleagues
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Read, write, and communicate prescription information clearly in English
- Explain dosage schedules and administration routes to patients
- Counsel patients on potential side effects and interactions
- Use the language of medication adherence and non-adherence

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can read a prescription aloud accurately, including dose, frequency, and route
- Can explain a medication regimen to a patient in clear lay language
- Can describe and explain the most important side effects of a given medication
- Can address non-adherence in a non-judgmental, supportive manner

Key Language & Vocabulary

- Prescription: Take [drug] [dose] [frequency] [route] / Once/twice/three times daily / With food
- Side effects: One thing to watch out for is... / Some people experience... / Rarely, this can cause...
- Adherence: It's important not to skip doses / What helps you remember to take it?
- Routes: by mouth / sublingually / topically / via injection / inhaled / intravenously

Session 23: Explaining Procedures and Obtaining Consent

Unit	UNIT 5: TREATMENT & MANAGEMENT
CEFR Level	B2
Competency	Procedure explanation and informed consent communication
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Explain a clinical procedure clearly and completely to a patient
- Describe what the patient will experience during the procedure
- Obtain valid informed consent using clear, accessible language
- Respond to patient concerns or refusals with respect and professionalism

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can explain a clinical procedure step-by-step in patient-friendly English
- Can clearly communicate the purpose, process, risks, and benefits of a procedure
- Can obtain informed consent using a structured, legally and ethically sound approach
- Can manage patient refusal or hesitation respectfully and professionally

Key Language & Vocabulary

- Explanation: We'd like to do a procedure called... / What this involves is... / You will feel...
- Consent: Before we proceed, I need to make sure you understand... / Do you have any questions?
- Risks & benefits: The main benefit is... / The key risks include... / Serious complications are rare but...
- Refusal: I completely respect your decision / Can you tell me more about your concerns?

Session 24: Post-Treatment Instructions & Follow-Up Care

Unit	UNIT 5: TREATMENT & MANAGEMENT
CEFR Level	B2
Competency	Patient education and post-treatment communication
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Give clear, structured post-treatment and discharge instructions in English
- Explain what to expect during recovery and when to seek further care
- Check patient comprehension using teach-back techniques
- Arrange follow-up appointments and communicate next steps clearly

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can deliver complete, structured post-treatment instructions verbally
- Can explain warning signs that require immediate medical attention
- Can use the teach-back technique to confirm patient comprehension
- Can communicate a clear follow-up plan including timing and responsible clinician

Key Language & Vocabulary

- Instructions: You should avoid... / It is important to... / Make sure you finish the full course
- Red flags: Please come back immediately if... / This is a warning sign: ... / Don't ignore...
- Teach-back: Can you tell me what you're going to do when you get home? / Just to check...
- Follow-up: I'd like to see you again in... / Your GP will contact you about... / We'll call with results

Session 25: Emergency Communication – SBAR and Rapid Handover

Unit	UNIT 5: TREATMENT & MANAGEMENT
CEFR Level	B2
Competency	Emergency clinical communication and escalation
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Use the SBAR framework to communicate clinical urgency clearly
- Give a rapid, structured verbal handover in an emergency setting
- Use language that communicates urgency without causing panic
- Request urgent help clearly and directly in clinical English

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can apply the SBAR framework to a deteriorating patient communication
- Can give a complete, structured emergency handover in under 90 seconds
- Can communicate clinical urgency with appropriate directness and precision
- Can ask for urgent help using assertive, professional escalation language

Key Language & Vocabulary

- SBAR: Situation, Background, Assessment, Recommendation
- Urgency cues: I'm calling about a patient I'm very concerned about / This is time-critical
- Assessment: My assessment is that... / I believe this may be... / The patient is deteriorating
- Recommendation: I think we need... / I'd like you to come and see this patient now
- Assertiveness: I need your help urgently / This cannot wait

UNIT 6: CLINICAL DOCUMENTATION & WRITING

Session 26: Writing SOAP Notes in English

Unit	UNIT 6: CLINICAL DOCUMENTATION & WRITING
CEFR Level	B1–B2
Competency	Clinical documentation – SOAP format
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Write clear, structured SOAP notes in English using appropriate clinical language
- Distinguish between subjective, objective, assessment, and plan documentation
- Use standard clinical abbreviations and terminology correctly in written notes
- Review and improve the clarity and completeness of a clinical note

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can write a complete SOAP note in English for a clinical encounter
- Can correctly categorise clinical information into S, O, A, and P sections
- Can use standard abbreviations correctly in written documentation
- Can identify and correct errors in a poorly written clinical note

Key Language & Vocabulary

- S: Patient reports... / Complains of... / Denies... / States that...
- O: On examination... / Vital signs reveal... / Investigations show...
- A: Impression: / Assessment: / Working diagnosis: / Differential includes...
- P: Plan: / To be started on... / For follow-up in... / Refer to... / Patient counselled re:

Session 27: Clinical Summaries and Case Reports

Unit	UNIT 6: CLINICAL DOCUMENTATION & WRITING
CEFR Level	B2
Competency	Professional medical writing – summaries and case reports
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Write a structured, concise clinical summary of a patient case
- Use passive constructions and formal tense conventions in medical writing
- Organise a clinical case report following standard academic structure
- Write clearly for a professional medical audience

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can produce a clear, logically structured clinical case summary
- Can use formal written medical English with appropriate passive constructions and tense
- Can organise a case report using: introduction, case presentation, discussion, conclusion
- Can identify and self-correct common errors in formal medical writing

Key Language & Vocabulary

- Passive voice: was admitted / was prescribed / was found to have / was referred for
- Formal transitions: On admission... / Subsequently... / Following this... / At discharge...
- Case report structure: The patient presented with... / Examination revealed... / A diagnosis of... was made
- Hedged conclusions: This case highlights... / This suggests that... / Further research is needed

Session 28: Referral Letters and Medical Correspondence

Unit	UNIT 6: CLINICAL DOCUMENTATION & WRITING
CEFR Level	B2
Competency	Professional written communication – referrals and letters
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Write a structured, professional referral letter in English
- Include all essential clinical information in the correct order
- Use appropriate formal language for different types of medical correspondence
- Write professionally to GPs, specialists, and other healthcare providers

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can write a complete, well-structured referral letter including all essential components
- Can select and present relevant clinical information efficiently in written form
- Can adapt tone and content for different types of recipient (GP, specialist, insurer)
- Can identify and correct errors of content, structure, and language in a referral letter

Key Language & Vocabulary

- Opening: I am writing to refer my patient... / I would be grateful if you could see...
- Background: She has a background of... / His relevant history includes...
- Reason for referral: I am concerned about... / She is being referred for further evaluation of...
- Closing: I would appreciate your expert opinion / Please do not hesitate to contact me

Session 29: Discharge Summaries

Unit	UNIT 6: CLINICAL DOCUMENTATION & WRITING
CEFR Level	B2
Competency	Professional medical writing – discharge documentation
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Write a complete, structured discharge summary in English
- Accurately document the admission diagnosis, treatment, and outcomes
- Clearly communicate ongoing management needs to the receiving clinician
- Use concise, precise clinical language in discharge documentation

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can write a complete discharge summary covering all essential components
- Can document the admission, investigations, treatment, and follow-up plan clearly
- Can write for a GP audience: complete but accessible
- Can identify gaps or errors in a given discharge summary

Key Language & Vocabulary

- Admission: [Patient] was admitted on [date] with a diagnosis of... / presenting with...
- Investigations: Investigations performed included... / CXR showed... / Blood results revealed...
- Treatment: Was treated with... / Underwent... / Responded well to... / No complications
- Discharge: Discharged on [date] / Current medications: / Follow-up arranged with... / GP to monitor

Session 30: Medical Terminology – Prefixes, Suffixes & Word Building

Unit	UNIT 6: CLINICAL DOCUMENTATION & WRITING
CEFR Level	B1–B2
Competency	Medical word structure and vocabulary expansion
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Identify and apply common medical prefixes, suffixes, and root words in English
- Deduce the meaning of unfamiliar medical terms using word structure
- Build and correctly use medical compound terms
- Distinguish between British and American English spelling variants in medical terms

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can correctly apply 20 high-frequency medical prefixes and suffixes
- Can decode unfamiliar medical terms using structural analysis
- Can construct accurate medical compound terms from given components
- Can identify common British/American spelling differences (e.g., haemoglobin/hemoglobin)

Key Language & Vocabulary

- Prefixes: tachy-, brady-, hyper-, hypo-, dys-, poly-, oligo-, a-/an-, peri-, endo-
- Suffixes: -itis, -ectomy, -plasty, -scopy, -algia, -ology, -pathy, -osis, -emia, -uria
- Roots: cardi, hepat, nephro, arthr, dermat, neuro, osteo, pneumo, haem/hem
- British vs. American: oedema/edema, anaemia/anemia, haematology/hematology

UNIT 7: PROFESSIONAL & INTERDISCIPLINARY COMMUNICATION

Session 31: Communicating with Colleagues – Handovers & Briefings

Unit	UNIT 7: PROFESSIONAL & INTERDISCIPLINARY COMMUNICATION
CEFR Level	B2
Competency	Interprofessional clinical communication
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Give a clear, complete, and efficient clinical handover to a colleague
- Participate in and lead a clinical briefing in English
- Use professional language when communicating clinical concerns to colleagues
- Manage interruptions, clarifications, and follow-up questions in a clinical handover

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can give a structured handover covering all essential patient information
- Can lead and contribute to a clinical briefing using appropriate professional language
- Can manage clarifying questions and follow-up during a verbal handover
- Can use the I-PASS handover tool fluently in English

Key Language & Vocabulary

- I-PASS: Illness severity, Patient summary, Action list, Situation awareness, Synthesis
- Briefing: The main concern for today is... / I want to flag... / We should watch out for...
- Clarification: Could you clarify what you mean by...? / Just to confirm...
- Handover: The plan is... / Outstanding issues include... / I'd want to know about...

Session 32: Case Presentations in English

Unit	UNIT 7: PROFESSIONAL & INTERDISCIPLINARY COMMUNICATION
CEFR Level	B2
Competency	Formal oral case presentation skills
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Structure and deliver a formal case presentation in English
- Use appropriate clinical language and organisation for a professional audience
- Manage questions and discussion following a case presentation
- Adapt a case presentation to different contexts (teaching round, grand round, case conference)

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can deliver a structured, fluent 5-minute case presentation in English
- Can use the standard case presentation sequence (ID, CC, HPC, PMH, SH, FH, E/O, IX, A, P)
- Can respond to clarifying or challenging questions during a case discussion
- Can adjust the level of detail in a presentation based on the audience and context

Key Language & Vocabulary

- Opening: This is a [age]-year-old [gender] who presented with... / I'm presenting a case of...
- Transitions: Moving on to the examination... / Of note on investigation... / In summary...
- Assessment: My impression is... / The working diagnosis is... / This is most consistent with...
- Handling questions: That's an excellent point / I'm not certain, but... / The evidence suggests...

Session 33: Multidisciplinary Team (MDT) Meetings

Unit	UNIT 7: PROFESSIONAL & INTERDISCIPLINARY COMMUNICATION
CEFR Level	B2
Competency	Collaborative decision-making in team settings
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Participate actively and appropriately in a simulated MDT meeting in English
- Present a patient case concisely for MDT discussion
- Contribute opinions and recommendations using professional language
- Manage disagreement, clarification, and consensus-building in a team setting

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can present a patient case concisely and clearly in an MDT context
- Can contribute professional opinions and recommendations during team discussion
- Can agree, politely disagree, and seek clarification in a team meeting
- Can summarise an MDT decision and document agreed actions

Key Language & Vocabulary

- Contributing: From my perspective as a... / In my department's view... / I'd suggest considering...
- Agreeing: I think that's the right approach / I'd support that plan / I agree with...
- Disagreeing politely: I see it slightly differently / I'd want to consider... / My concern would be...
- Summarising: So we've agreed that... / The MDT recommendation is... / The action points are...

Session 34: Telehealth & Remote Patient Communication

Unit	UNIT 7: PROFESSIONAL & INTERDISCIPLINARY COMMUNICATION
CEFR Level	B2
Competency	Digital and remote consultation skills
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Conduct effective patient consultations via telephone and video in English
- Manage the communicative challenges specific to remote consultations
- Check patient comprehension and establish non-verbal connection remotely
- Document a telehealth consultation appropriately

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can open, conduct, and close a telephone or video consultation professionally
- Can compensate for the absence of non-verbal cues using verbal strategies
- Can manage technical difficulties, silence, and misunderstanding in remote consultations
- Can document a remote consultation including the modality used and any limitations

Key Language & Vocabulary

- Opening: Hi, this is Dr [name] calling from [clinic] / Can you hear me clearly?
- Clarification: Could you say that again? / I'm just going to mute myself while you speak
- Compensating for no visual: Can you describe exactly what you're seeing? / Since I can't see you...
- Closing: I'll send you a summary by email / Please call us if anything changes

Session 35: Reading & Discussing Medical Literature

Unit	UNIT 7: PROFESSIONAL & INTERDISCIPLINARY COMMUNICATION
CEFR Level	B2
Competency	Academic English and evidence-based medicine
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Read and comprehend a medical research abstract in English
- Identify and discuss key components of a research article (method, findings, limitations)
- Critically evaluate evidence quality using appropriate academic language
- Discuss clinical implications of a study using evidence-based language

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can identify and summarise the key components of a medical research abstract
- Can discuss research findings using appropriate academic hedging
- Can critically evaluate research quality using relevant criteria
- Can apply study findings to a clinical scenario and justify using evidence-based language

Key Language & Vocabulary

- Study components: abstract, methodology, intervention, control group, primary outcome, p-value
- Hedging: This study suggests... / The authors found evidence of... / These results indicate...
- Critical evaluation: The main limitation is... / The generalisability of these findings...
- Clinical application: Based on this evidence, we might consider... / The implication for practice is...

UNIT 8: COMPLEX & SENSITIVE COMMUNICATION

Session 36: Breaking Bad News – The SPIKES Protocol

Unit	UNIT 8: COMPLEX & SENSITIVE COMMUNICATION
CEFR Level	B2
Competency	Sensitive communication and emotional support in critical conversations
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Apply the SPIKES protocol to deliver serious news sensitively in English
- Use language that is honest, compassionate, and clearly understood by the patient
- Respond to emotional reactions (shock, anger, denial, grief) during a bad news consultation
- Provide appropriate support and next-step information after delivering bad news

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can conduct a bad news consultation using the SPIKES framework
- Can use language that is honest yet compassionate and appropriately paced
- Can respond to a range of emotional reactions with empathy and professional composure
- Can provide a clear, manageable plan and emotional support following bad news

Key Language & Vocabulary

- SPIKES: Setting, Perception, Invitation, Knowledge, Emotions, Strategy
- Warning shot: I have some news that I think may be difficult to hear
- Delivering: I'm afraid the results show... / Unfortunately, the tests confirm...
- Support: I know this is a lot to take in / You don't have to go through this alone / We're here to help

Session 37: Informed Consent, Patient Rights & Shared Decision-Making

Unit	UNIT 8: COMPLEX & SENSITIVE COMMUNICATION
CEFR Level	B2
Competency	Ethical communication and patient autonomy
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Explain patient rights using clear, accessible English
- Conduct a complete informed consent process for a complex procedure
- Support patient autonomy and independent decision-making through language choices
- Respond to refusal of treatment respectfully and ethically

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can explain the legal and ethical basis for informed consent in clear language
- Can conduct a complete, valid consent process for a complex medical scenario
- Can use language that respects and promotes patient autonomy
- Can manage and document patient refusal professionally and ethically

Key Language & Vocabulary

- Rights language: You have the right to... / You are entitled to a second opinion / You can change your mind
- Capacity: Do you understand what I've explained? / Can you tell me in your own words?
- Refusal: I completely respect your decision / May I ask what's making you hesitant?
- Documentation: Patient informed of risks and benefits / Patient declined treatment after discussion

Session 38: Cross-Cultural Communication in Medicine

Unit	UNIT 8: COMPLEX & SENSITIVE COMMUNICATION
CEFR Level	B2
Competency	Cultural sensitivity and effective intercultural healthcare communication
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Identify potential cultural differences that affect medical communication in English
- Adapt communication style to respect patients' cultural backgrounds
- Use culturally sensitive language when discussing sensitive topics
- Navigate language barriers and work effectively with interpreters

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can identify key cultural dimensions that affect clinical communication
- Can adapt history-taking and explanation language for cultural sensitivity
- Can work effectively with a professional medical interpreter
- Can avoid cultural assumptions and apply patient-centred, individuality-respecting language

Key Language & Vocabulary

- Checking for assumptions: I'd like to understand how you see this / What does health mean to you?
- Exploring beliefs: Is there anything in your beliefs or values that might affect your care?
- Interpreter language: I'll be speaking through an interpreter / Please speak directly to the patient
- Checking understanding: Different people understand this differently – what makes sense to you?

Session 39: Ethics, Confidentiality & Professional Boundaries in English

Unit	UNIT 8: COMPLEX & SENSITIVE COMMUNICATION
CEFR Level	B2
Competency	Professional and ethical communication in English
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Discuss key medical ethics principles (autonomy, beneficence, non-maleficence, justice) in English
- Explain confidentiality obligations and exceptions to patients and colleagues
- Use appropriate professional language when addressing ethical concerns
- Navigate professional boundary discussions using respectful, clear English

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can explain the 4 principles of medical ethics in accessible language
- Can clearly communicate confidentiality policies to patients
- Can discuss an ethical dilemma professionally using structured reasoning language
- Can address professional boundary issues using firm but respectful English

Key Language & Vocabulary

- Ethics principles: patient autonomy, best interests, 'do no harm', fairness of access
- Confidentiality: Everything you tell me is confidential / There are some exceptions to this
- Dilemma language: On one hand... / However, we also have a duty to... / A possible compromise would be...
- Boundaries: I understand your request, but as your doctor, I'm not able to... / My role is to...

Session 40: Integrated Review, Reflection & Mock Consultation Assessment

Unit	UNIT 8: COMPLEX & SENSITIVE COMMUNICATION
CEFR Level	B2
Competency	Integrated medical English communication – all competencies
Duration	60 minutes

Learning Objectives

By the end of this session, learners will be able to:

- Demonstrate integrated use of all major consultation and communication skills from the course
- Self-reflect on learning progress and identify remaining language development goals
- Perform a full simulated clinical consultation in English for assessment
- Give and receive structured, constructive feedback on clinical communication performance

Learning Outcomes

Upon successful completion of this session, learners will demonstrate:

- Can conduct a complete, integrated clinical consultation in English
- Can identify personal strengths and areas for further development in medical English
- Can give specific, evidence-based feedback to a peer on their consultation performance
- Can set a personal post-course language development action plan

Key Language & Vocabulary

- Feedback language: One strength I noticed was... / An area to develop further would be...
- Reflection: I felt most confident when... / I still find it challenging to... / My goal now is...
- Self-assessment: On a scale of 1 to 5, I rate myself... because...
- Goal setting: By [date], I aim to... / I will practise... / I will measure my progress by...

PART THREE: INSTRUCTIONAL GUIDE FOR TEACHERS

This section provides detailed, step-by-step teaching guidance for each session. Each entry specifies how to deliver the lesson, timing for each phase, suggested activities, materials required, and practical teaching notes. These guides are designed for English language teachers who do not necessarily have a medical background.

Author: Rayene Medabis

UNIT 1: MEDICAL FOUNDATIONS

Session 1: Introduction to Medical English & Professional Identity

CEFR Level B1	Duration 60 minutes	Competency	Professional self-expression and introductions
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Introduce themselves professionally in English using key career information
- Describe their medical role, specialization, and work context
- Understand the course structure and identify personal language learning goals
- Use basic professional vocabulary related to medicine and healthcare settings

Outcomes – Learners will demonstrate:

- Can deliver a clear, structured 60-second professional self-introduction
- Can describe their medical specialty and daily responsibilities in English
- Can articulate 2–3 specific language goals for the course
- Can participate in a basic peer discussion about professional backgrounds

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Ice-breaker: 'Two Truths and a Medical Myth'. Each doctor writes two true professional facts and one false one. Pairs guess the myth. Elicits vocabulary naturally and reduces anxiety.

Presentation ~15 min

15 min – Introduce the PARROT framework (Position, Area, Responsibilities, Role, Organization, Training). Board-write key phrases. Play a short audio of a doctor introducing themselves at a conference. Highlight stress patterns in medical titles.

Guided Practice ~15 min

15 min – Pair work using guided question cards. Doctor A interviews Doctor B following the PARROT structure. Swap. Teacher monitors and notes common errors for group feedback.

Freer Practice / Production

15 min – Each doctor delivers a 60-second introduction to the group. Peers use a three-item observation checklist: (1) Was the role clear? (2) Were

~15 min

sentences complete? (3) Was the pace comfortable?

Wrap-Up

~5 min

5 min – Class reflection on communication challenges. Distribute and complete a personal goal-setting worksheet. Preview Session 2.

Materials Required

1. PARROT framework handout
2. Question card sets (A & B roles)
3. Peer observation checklist
4. Personal goal-setting worksheet
5. Audio clip: medical conference introduction (2 min)

Teacher Notes

Set a relaxed, validating tone. Many doctors feel self-conscious about English. Correct errors indirectly in this first session. Focus on fluency over accuracy. Use elicitation rather than presentation of answers where possible.

Session 2: The Human Body – Anatomy Vocabulary in Context

CEFR Level B1	Duration 60 minutes	Competency	Medical vocabulary and anatomical description
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Name major anatomical structures and body systems using correct English terminology
- Use directional anatomical terms (superior, inferior, medial, lateral, proximal, distal)
- Describe the location and function of key organs in English
- Distinguish between medical terminology and lay language for common structures

Outcomes – Learners will demonstrate:

- Can correctly label and describe major anatomical structures
- Can use anatomical directional language in descriptive sentences
- Can explain organ location to a patient using accessible lay language
- Can switch appropriately between medical register and patient-friendly terms

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Unlabelled body diagram race: pairs have 3 minutes to label as many anatomical parts as they can. Score and review. Establishes vocabulary baseline.

Presentation ~15 min

15 min – Introduce directional terms using a large body chart. Model descriptions of organ locations ('The liver is located in the right upper quadrant of the abdomen, inferior to the diaphragm'). Contrast with patient-friendly language.

Guided Practice ~15 min

15 min – 'Where is it?' activity: teacher describes an organ location; students point to it. Then students take turns describing to a partner who locates it on the diagram. Swap roles.

Freer Practice / Production ~15 min

15 min – Roleplay: Doctor (A) must explain to a 'patient' (B) where a condition is in the body using lay terms only. B asks clarifying questions. Groups of 3 with an observer.

Wrap-Up ~5 min

5 min – Vocabulary journal: each doctor records 5 new terms with their lay equivalent. Quick 10-term verbal quiz.

Materials Required

1. Labelled and unlabelled anatomical diagrams
2. Directional terms reference card

3. Vocabulary journal template

4. Quiz slip (10 terms)

Teacher Notes

Do not re-teach anatomy – doctors know this. Focus entirely on English expression. Use their knowledge as an asset. Reward when they switch registers successfully.

Session 3: Medical Specialties, Roles & Healthcare Settings

CEFR Level B1	Duration 60 minutes	Competency	Professional and institutional vocabulary
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Name and describe major medical specialties and their clinical scope
- Identify and describe professional roles within a healthcare team
- Describe different healthcare settings and their functions
- Ask and answer questions about professional roles in English

Outcomes – Learners will demonstrate:

- Can accurately describe their own specialty and those of colleagues
- Can explain the roles of doctors, nurses, allied health, and support staff
- Can describe healthcare settings including hospital departments and primary care
- Can engage in small-talk conversation about professional backgrounds

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – 'Specialty Bingo': each learner has a bingo card with specialty names. Teacher reads definitions; learners cross off the matching specialty. First to complete a row wins.

Presentation ~15 min

15 min – Present a hospital organisation chart on the board. Explain each role and department. Model sentences describing team structure and individual roles. Teach the language of hierarchy and collaboration.

Guided Practice ~15 min

15 min – Matching activity: role cards matched to description cards. Then pair discussion: 'Describe your team to your partner without using the specialty name.'

Freer Practice / Production ~15 min

15 min – Group task: teams design and present a fictional 'ideal hospital department', naming all roles and responsibilities. Other groups ask questions.

Wrap-Up ~5 min

5 min – Class compiles a 'Quick Reference' vocabulary card of specialties and roles. Exit ticket: write two sentences about their own team.

Materials Required

1. Specialty Bingo cards
2. Hospital organisation chart handout
3. Role-description matching cards

4. Group task instruction sheet

5. Quick Reference card template

Teacher Notes

Elicit vocabulary from learners first – they likely know many terms. Highlight British vs. American English differences (e.g., paediatrics / pediatrics, theatre / OR). Discuss the social register used when speaking to different team members.

Session 4: Numbers, Measurements & Medical Data

CEFR Level B1	Duration 60 minutes	Competency	Numerical and quantitative communication in medicine
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Say, write, and comprehend numbers, decimals, fractions, and percentages in medical contexts
- Communicate measurements such as height, weight, dosage, and lab values
- Use correct language to describe ranges, trends, and changes in data
- Interpret and report basic clinical data verbally and in writing

Outcomes – Learners will demonstrate:

- Can accurately say and write medical numbers, measurements, and dosages
- Can describe data trends using appropriate language (elevated, decreased, within normal limits)
- Can report a patient's vitals and basic laboratory values clearly
- Can interpret and communicate simple statistical data from a medical context

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Number dictation: teacher reads 10 medical figures (e.g., 'one point three five milligrams per decilitre'). Learners write them. Review common transcription errors.

Presentation ~15 min

15 min – Introduce medical number language: reading decimals, fractions, units of measure, and dosages. Teach trend language. Use a sample vital signs chart to model verbal reporting.

Guided Practice ~15 min

15 min – Pair work: one learner reads a lab results table aloud; partner writes what they hear. Then they compare and discuss discrepancies.

Freer Practice / Production ~15 min

15 min – Scenario: each doctor receives a simulated patient results sheet and must give a 2-minute verbal summary to a colleague, including all relevant figures and trends.

Wrap-Up ~5 min

5 min – Class correction of common errors noted during production. Review of the most important number patterns in clinical English.

Materials Required

1. Number dictation handout
2. Sample vital signs chart
3. Lab results table (pair activity)

4. Simulated patient results sheets

5. Trend language reference card

Teacher Notes

Pay close attention to pronunciation of decimals (point vs. full stop), units (mg vs. mcg), and large numbers. These are common sources of medical errors. Emphasise clarity and confirmation-seeking ('Did you say one point three or thirteen?').

Session 5: Professional Register – Formal vs. Informal Language in Medicine

CEFR Level B1–B2	Duration 60 minutes	Competency	Register awareness and code-switching in medical communication
-----------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Distinguish between formal, informal, and technical registers in medical English
- Adapt language appropriately for different audiences (patients, colleagues, superiors)
- Identify and avoid jargon when communicating with non-medical audiences
- Use appropriate hedging and softening language in professional contexts

Outcomes – Learners will demonstrate:

- Can identify the appropriate register for a given medical communication context
- Can reformulate technical medical language into accessible patient-friendly explanations
- Can use formal professional language when writing or speaking to superiors
- Can soften directives and requests appropriately (Could you... / It would be helpful if...)

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Sort it: learners receive 15 sentence cards and sort them into three piles: 'formal/professional', 'patient-friendly', 'too informal'. Discuss borderline cases.
Presentation ~15 min	15 min – Explain the concept of register and code-switching. Show the same clinical scenario written in three registers. Discuss when each is appropriate. Introduce hedging and softening strategies.
Guided Practice ~15 min	15 min – Reformulation drill: teacher gives a clinical sentence; learners reformulate it for (a) a colleague, (b) a patient, (c) a supervisor. Discuss choices.
Freer Practice / Production ~15 min	15 min – Roleplay: the same doctor must explain a diagnosis first to a colleague (using medical terminology) then to the patient (using lay language). Observer notes language differences.
Wrap-Up ~5 min	5 min – Build a 'Register Spectrum' chart on the board together. Assign homework: identify 3 medical sentences from their practice and reformulate each for a different audience.

Materials Required

1. Register sort cards (15 cards)
2. Three-register scenario comparison sheet
3. Reformulation drill cards
4. Roleplay scenario sheets
5. Register Spectrum chart template

Teacher Notes

Doctors often over-medicalise speech even with patients, or under-formalise with superiors. This session addresses both extremes. Normalise hedging language as a professional skill, not a sign of uncertainty.

UNIT 2: OPENING THE CONSULTATION

Session 6: Opening Consultations & Establishing Rapport

CEFR Level B1	Duration 60 minutes	Competency	Patient-centred consultation opening skills
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Use appropriate greetings, introductions, and small-talk to open a consultation
- Establish rapport and build trust through verbal and linguistic strategies
- Demonstrate active interest using listener responses and follow-up questions
- Use open-ended questions to invite the patient to share their reason for visiting

Outcomes – Learners will demonstrate:

- Can open a medical consultation professionally and warmly in English
- Can use small-talk and reassuring language to put patients at ease
- Can use open-ended questions to invite the patient's narrative ("What brings you in today?")
- Can use listener responses (I see / Go on / Tell me more) to encourage the patient

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Watch 2 short video clips of consultation openings (one poor, one excellent). Learners identify what made each good or poor. Introduces the concept of patient-centred communication.
Presentation ~15 min	15 min – Present the ICE framework (Ideas, Concerns, Expectations). Demonstrate a consultation opening, narrating the strategies used. Highlight listener responses and open questions.
Guided Practice ~15 min	15 min – Dialogue completion: learners fill in the gaps in a consultation transcript using provided language. Then read the completed dialogue aloud with a partner.
Freer Practice / Production ~15 min	15 min – Roleplay in pairs: one plays the doctor, one plays the patient (using given role cards). Focus on the opening 3 minutes only. Observer gives feedback on rapport-building.
Wrap-Up ~5 min	5 min – Class discussion: What barriers to rapport do you face with English-speaking patients? Consolidate key phrases on the board.

Materials Required

1. Video clips (or transcripts) of consultation openings
2. ICE framework handout
3. Dialogue completion worksheet
4. Patient role cards
5. Observer feedback sheet

Teacher Notes

Some doctors may find rapport-building language awkward. Emphasise that small warmth cues carry significant clinical impact. If video is unavailable, use audio transcripts or teacher demonstration.

Session 7: Eliciting the Chief Complaint

CEFR Level B1	Duration 60 minutes	Competency	Structured history taking – presenting complaint
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Use appropriate language to ask about and elicit a patient's chief complaint
- Encourage the patient to describe the problem in their own words
- Identify and use follow-up probing questions
- Clarify and summarise the presenting complaint back to the patient

Outcomes – Learners will demonstrate:

- Can use open and closed questions strategically to explore the chief complaint
- Can invite a patient narrative without interrupting prematurely
- Can use probing follow-up questions to deepen understanding
- Can accurately summarise the chief complaint back to the patient for confirmation

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – 'What's wrong with this doctor?' – read or listen to a consultation where the doctor interrupts, uses closed questions only, and jumps to conclusions. Learners identify the errors.
Presentation ~15 min	15 min – Introduce the Calgary-Cambridge model of consultation. Focus on 'gathering information'. Model the technique of eliciting a patient narrative with think-aloud commentary.
Guided Practice ~15 min	15 min – Question transformation drill: convert 10 closed questions to open-ended equivalents. Then arrange question types in the order they should appear in a real consultation.
Freer Practice / Production ~15 min	15 min – Roleplay: doctor must elicit the full chief complaint without using a single closed question for the first 2 minutes. Then use clarifying and summarising language.
Wrap-Up ~5 min	5 min – Exit ticket: learners write 3 open questions and 1 summarising phrase they would use in their own practice.

Materials Required

1. Flawed consultation transcript
2. Calgary-Cambridge model overview

3. Question transformation worksheet

4. Roleplay scenario cards

5. Exit ticket slips

Teacher Notes

The 'golden minute' concept is key here: letting patients speak uninterrupted for the first 60 seconds of a consultation dramatically improves outcomes. Highlight this as an evidence-based communication strategy.

Session 8: Describing and Asking About Symptoms

CEFR Level B1	Duration 60 minutes	Competency	Symptom elicitation and clinical vocabulary
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Identify and use clinical vocabulary for common symptoms across body systems
- Ask patients to describe symptoms in both lay and medical language
- Describe symptoms using appropriate adjectives (severity, character, quality)
- Distinguish between symptoms reported by patients and signs observed by doctors

Outcomes – Learners will demonstrate:

- Can name and describe common symptoms in both medical and lay language
- Can ask patients to elaborate on symptoms using appropriate descriptive questions
- Can use symptom-describing adjectives correctly (dull, sharp, burning, throbbing)
- Can distinguish between subjective symptoms and objective clinical signs in English

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Symptom vocabulary brainstorm: 'List every symptom you might hear from a patient in a typical week.' Group mind-map on the board. Teacher adds gaps.

Presentation ~15 min

15 min – Introduce the SOCRATES mnemonic briefly (to be expanded in Session 9). Focus today on character and quality descriptors. Use image cards showing faces expressing different types of pain.

Guided Practice ~15 min

15 min – Pair matching: symptom images with clinical terms, then with lay descriptions. Then a gap-fill transcript: patient describes symptoms in lay terms; learner writes the medical term.

Freer Practice / Production ~15 min

15 min – Doctor-patient roleplay: patient describes a set of symptoms (from role card) in lay terms; doctor must elicit more information and record symptoms using medical vocabulary.

Wrap-Up ~5 min

5 min – Class reviews the most common errors in symptom vocabulary. Highlight 5 frequently confused pairs (e.g., dyspnoea vs. orthopnoea, nausea vs. vomiting).

Materials Required

1. Symptom vocabulary image cards

2. SOCRATES preview handout

3. Gap-fill transcript worksheet

4. Roleplay symptom cards

5. Commonly confused pairs list

Teacher Notes

Focus on the bridge between what patients say and what doctors document. The ability to translate lay language to clinical language is a key competency. Avoid overwhelming learners with too many new medical terms – reinforce high-frequency ones.

Session 9: Exploring Pain – The SOCRATES Framework

CEFR Level B1–B2	Duration 60 minutes	Competency	Structured pain assessment and communication
-----------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Apply the SOCRATES framework to conduct a structured pain assessment
- Use a full range of questions to explore each dimension of pain
- Record pain information accurately using clinical language
- Explain pain assessment to a patient in accessible terms

Outcomes – Learners will demonstrate:

- Can systematically explore pain using all 8 SOCRATES dimensions
- Can generate 2–3 questions for each SOCRATES category
- Can document pain findings clearly in a clinical note
- Can explain to a patient why and how their pain is being assessed

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Quick recall quiz: learners try to name all 8 SOCRATES components from memory (introduced last session). Discuss why each matters clinically.

Presentation ~15 min

15 min – Model a full pain history using SOCRATES. Play role of doctor and patient with a co-teacher or volunteer. Narrate each question's purpose.

Guided Practice ~15 min

15 min – SOCRATES question-building: for each category, teams generate 3 questions. Present to class. Teacher evaluates for clarity and clinical appropriateness.

Freer Practice / Production ~15 min

15 min – Full SOCRATES roleplay: doctor conducts a structured pain assessment; patient plays a specific pain scenario (e.g., chest pain, headache, abdominal pain). Observer uses SOCRATES checklist.

Wrap-Up ~5 min

5 min – Debrief: what SOCRATES components do learners find hardest to ask about in English? Focus correction on those areas.

Materials Required

1. SOCRATES reference card
2. Question-building task sheets
3. Roleplay pain scenario cards

4. SOCRATES observation checklist
5. Clinical note documentation template

Teacher Notes

This is a cornerstone clinical communication session. Ensure every learner gets to practice as both doctor and patient. Reinforce that these questions should flow naturally, not feel like a checklist recitation.

Session 10: Active Listening & Empathic Language

CEFR Level B2	Duration 60 minutes	Competency	Emotional intelligence and patient-centred communication
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Identify and apply verbal active listening strategies in medical consultations
- Use empathic responses to acknowledge patients' emotions and concerns
- Respond appropriately to unexpected patient disclosures or emotional moments
- Balance clinical efficiency with emotional responsiveness

Outcomes – Learners will demonstrate:

- Can use verbal active listening strategies (reflection, paraphrasing, silence) effectively
- Can identify patient emotions from speech and respond empathically
- Can respond to patient distress using the NURSE framework (Name, Understand, Respect, Support, Explore)
- Can maintain clinical progress while responding to emotional content

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Audio vignette: listen to a 90-second patient describing an emotionally charged situation. Learners note the emotions expressed. Discuss what an ideal doctor response would sound like.

Presentation ~15 min

15 min – Introduce the NURSE framework. Show video or demonstrate contrasting responses to a patient in distress (dismissive vs. empathic). Discuss clinical and ethical reasons for empathic communication.

Guided Practice ~15 min

15 min – Empathy rewrite: give learners 8 dismissive or neutral doctor responses; rewrite them using empathic language. Share and compare rewrites.

Freer Practice / Production ~15 min

15 min – Emotionally charged roleplay: patient reveals anxiety, grief, or fear during consultation. Doctor must respond empathically using NURSE before continuing clinically.

Wrap-Up ~5 min

5 min – Reflection: 'What is the hardest emotion to respond to in English?' Discuss cultural and linguistic barriers to empathy. Set the norm: empathy is a skill, not just a personality trait.

Materials Required

1. Audio vignette recording or transcript

2. NURSE framework handout
3. Empathy rewrite worksheet
4. Emotionally charged roleplay cards
5. Reflection discussion prompts

Teacher Notes

This session often surfaces personal and cultural reflections on emotional expression in medical settings. Allow space for this. Reassure learners that language barriers do not have to prevent empathic communication.

UNIT 3: TAKING A COMPLETE HISTORY

Session 11: Past Medical History, Medications & Allergies

CEFR Level B1–B2	Duration 60 minutes	Competency	Systematic history taking – past history
-----------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Ask systematic questions about a patient's past medical and surgical history
- Elicit accurate medication histories including dosages and adherence
- Inquire about allergies and document adverse reactions appropriately
- Use structured PMH/Medication/Allergy language in clinical documentation

Outcomes – Learners will demonstrate:

- Can systematically elicit a past medical and surgical history
- Can document medications with name, dose, frequency, and route
- Can distinguish between allergies and intolerances and ask about both
- Can document PMH, medications, and allergies in standard clinical format

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Error-spotting: show a poorly completed PMH section of a clinical note. Learners identify missing or unclear information and discuss why each element matters.
Presentation ~15 min	15 min – Walk through a structured PMH framework. Demonstrate the full sequence of questions. Highlight the clinical importance of each (e.g., why adherence matters, why reaction type matters for allergies).
Guided Practice ~15 min	15 min – Simulated patient case: learners read a complex patient history then role-play extracting it through questions only (no reading allowed for the patient).
Freer Practice / Production ~15 min	15 min – Full PMH roleplay followed by written documentation: doctor elicits the PMH, then writes it up in correct clinical format. Partner checks accuracy.
Wrap-Up ~5 min	5 min – Review the 5 most common documentation errors. Discuss the clinical risks of incomplete medication or allergy recording.

Materials Required

1. Flawed clinical note handout
2. PMH framework reference card
3. Simulated patient case files
4. PMH documentation template
5. Common documentation errors list

Teacher Notes

Emphasise that this is not about memorising a checklist but about understanding the clinical reason for each question. Many doctors in non-English settings may use abbreviated or non-standard formats – bridge to English clinical standards.

Session 12: Family History & Genetic Risk Communication

CEFR Level B2	Duration 60 minutes	Competency	Genetic risk elicitation and patient communication
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Elicit a structured family history relevant to the patient's presenting complaint
- Communicate genetic risk information clearly and sensitively
- Use kinship terminology accurately in English
- Explain hereditary conditions and risk factors in lay language

Outcomes – Learners will demonstrate:

- Can construct a structured family history using appropriate English questions
- Can use correct kinship terms (first-degree relative, maternal, paternal) accurately
- Can explain hereditary risk to a patient using accessible, non-alarming language
- Can document family history clearly in a clinical note or pedigree format

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Kinship terminology quiz: learners fill in a family tree with the correct English terms for each relationship. Review and discuss edge cases (e.g., half-sibling, step-parent).

Presentation ~15 min

15 min – Demonstrate a full family history elicitation. Then model how to explain genetic risk to a patient: acknowledge uncertainty, avoid catastrophising, focus on actionable information.

Guided Practice ~15 min

15 min – Role-play pairs: elicit a family history from a prepared patient profile. Focus on completeness (three generations) and sensitivity.

Freer Practice / Production ~15 min

15 min – Scenario: patient has just been told there is a family history of BRCA mutation. Doctor must explain what this means and what the next steps are. Focus on the language of risk and reassurance.

Wrap-Up ~5 min

5 min – Class discussion: What English vocabulary do you find hardest when discussing family history? Which terms do your patients misunderstand?

Materials Required

1. Family tree diagram worksheet
2. Kinship terms reference card
3. Patient profile cards

4. BRCA communication scenario

5. Risk language reference sheet

Teacher Notes

This session bridges clinical genetics and communication. Emphasise the 'non-directive' principle: communicating risk without inducing unnecessary panic. Correct over-confident risk statements ('you will get cancer') and model probabilistic language.

Session 13: Social History & Lifestyle Factors

CEFR Level B2	Duration 60 minutes	Competency	Lifestyle and social determinants elicitation
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Elicit a sensitive and comprehensive social history in English
- Ask about smoking, alcohol, recreational drugs, and sexual history appropriately
- Explore occupation, living situation, and social supports
- Use non-judgmental, respectful language when discussing sensitive lifestyle topics

Outcomes – Learners will demonstrate:

- Can ask about all major social history components using appropriate, respectful English
- Can calculate and communicate pack-year history and alcohol units accurately
- Can use non-judgmental framing for sensitive questions (e.g., drug use, sexual history)
- Can integrate social history into a holistic clinical picture

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Sensitive question warm-up: which of these 10 questions would feel awkward to ask in English? Vote and discuss. Surfaces anxiety and establishes purpose of the session.

Presentation ~15 min

15 min – Structured overview of the social history: HEADS (Home, Education/Employment, Activities, Drugs, Sex). Demonstrate non-judgmental question framing with examples. Model pack-year calculation.

Guided Practice ~15 min

15 min – Question reformulation: take 8 judgmental or awkward questions and reformulate them using appropriate framing. Discuss in pairs, then share with class.

Freer Practice / Production ~15 min

15 min – Full social history roleplay including a sensitive topic (alcohol misuse or drug use). Observer uses a structured feedback sheet focusing on language tone and completeness.

Wrap-Up ~5 min

5 min – Reflection: what language strategies help normalise sensitive questions? Consolidate the 'I ask everyone this' technique and universalising language.

Materials Required

1. Sensitive question discussion cards

2. HEADS framework handout
3. Question reformulation worksheet
4. Social history roleplay cards with observer sheet
5. Pack-year calculation guide

Teacher Notes

Be culturally sensitive. What is considered 'sensitive' varies by background. Acknowledge that these conversations may feel different in English vs. the learner's first language. Validate that this is a real clinical challenge.

Session 14: Review of Systems in English

CEFR Level B2	Duration 60 minutes	Competency	Systematic ROS elicitation and documentation
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Conduct a systematic review of systems (ROS) in English
- Use appropriate screening questions for each body system
- Distinguish between positive and negative findings in ROS
- Document ROS findings using standard clinical abbreviations and terminology

Outcomes – Learners will demonstrate:

- Can ask at least 2 screening questions per major body system
- Can conduct a complete ROS in under 5 minutes using efficient clinical English
- Can document both positive and pertinent negative findings appropriately
- Can use standard abbreviations correctly (SOB, LOC, CP, N/V, etc.)

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – System identification challenge: teacher gives a symptom; learners name the system it belongs to. Builds vocabulary associations. Race format.

Presentation ~15 min

15 min – Present the ROS by system. Model rapid screening questions for each (aiming for 1–2 per system in a real consultation). Show how to document positive and negative findings.

Guided Practice ~15 min

15 min – ROS worksheet: complete a standardised ROS form based on a written patient case. Discuss answers in pairs.

Freer Practice / Production ~15 min

15 min – Timed ROS roleplay: doctors must complete a 5-system ROS in 4 minutes and document findings in real time. Observer checks for missing systems.

Wrap-Up ~5 min

5 min – Group creates a single-page 'ROS Quick Reference' card together. Exit ticket: write the 2 most important screening questions for any 3 systems.

Materials Required

1. ROS question reference list by system
2. ROS documentation worksheet
3. Patient case for practice
4. Timed roleplay cards

5. Observer checklist**6. ROS Quick Reference template****Teacher Notes**

Prioritise efficiency and coverage. Doctors can be verbose; help them develop a fast, systematic approach. Emphasise the clinical value of pertinent negatives – these are as important as positives.

Session 15: Pediatric & Geriatric Communication Adaptations

CEFR Level B2	Duration 60 minutes	Competency	Adapting consultation language for specific patient populations
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Adapt history-taking language for paediatric patients and their parents/carers
- Adjust communication for elderly patients, accounting for cognitive and sensory needs
- Use age-appropriate vocabulary and explanations when speaking with children
- Manage three-way communication between doctor, patient, and carer

Outcomes – Learners will demonstrate:

- Can communicate directly with children using age-appropriate English
- Can include parents/carers in the consultation without excluding the child
- Can adapt pacing, vocabulary, and approach for elderly or cognitively impaired patients
- Can manage the dynamics of a three-way consultation in English

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Short video: a consultation where the doctor speaks only to the parent and ignores the child. Learners identify what went wrong and what should have been different.

Presentation ~15 min

15 min – Two mini-lectures: (1) paediatric communication: age-specific adaptations, using toys/drawings, engaging the child while informing the carer. (2) Geriatric communication: speech rate, clarity, checking comprehension, managing cognitive impairment.

Guided Practice ~15 min

15 min – Script rewriting: rewrite two consultation excerpts – one paediatric, one geriatric – to be more appropriate. Compare with a partner.

Freer Practice / Production ~15 min

15 min – Three-way roleplay: doctor, patient (child or elderly), and carer. Doctor must balance both. Observer notes whether the patient was heard and included.

Wrap-Up ~5 min

5 min – Key adaptations summary. Discussion: 'What are the most common mistakes doctors make with these patient groups?'

Materials Required

1. Video clip (or transcript) of doctor-parent consultation
2. Age adaptation reference cards

3. Script rewriting worksheets
4. Three-way roleplay scenario packs
5. Observer checklist

Teacher Notes

Remind learners that the ethical principle of involving the patient directly applies at all ages. Avoid letting the carer dominate. For geriatric patients, emphasise the dignity-preserving function of slow, clear, respectful language.

UNIT 4: CLINICAL EXAMINATION & DIAGNOSIS

Session 16: The Physical Examination – Instructions & Vocabulary

CEFR Level B1–B2	Duration 60 minutes	Competency	Giving clear examination instructions and describing findings
-----------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Give clear, polite instructions during a physical examination in English
- Describe physical examination findings using appropriate clinical terminology
- Respond to patient questions or discomfort during the examination
- Use appropriate hedging and permission-seeking before and during examination

Outcomes – Learners will demonstrate:

- Can give a full set of examination instructions clearly and politely
- Can describe examination findings using clinical terms and lay equivalents
- Can address patient concerns or discomfort during examination appropriately
- Can seek permission before each step of a physical examination

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – 'Simon Says' medical edition: teacher gives examination instructions; learners perform actions. Quickly energises and establishes the imperative/polite instruction forms.
Presentation ~15 min	15 min – Demonstrate a structured physical examination verbally. Highlight the difference between instructions (to patient) and findings descriptions (to colleague). Show hedged vs. blunt forms.
Guided Practice ~15 min	15 min – Instruction transformation: convert direct commands into polite clinical requests. Then match examination instructions to the correct examination step.
Freer Practice / Production ~15 min	15 min – Physical examination roleplay: doctor must conduct a verbal examination of a specific system (cardiovascular, respiratory, or abdominal), giving all instructions and narrating findings aloud.
Wrap-Up ~5 min	5 min – Class generates a 'cheat sheet' of 15 most common examination instructions. Homework: practise instructions for a complete neurological exam.

Materials Required

1. Examination instruction transformation cards
2. Examination step-matching activity
3. System roleplay scenario cards
4. Observer feedback form
5. 15 common instructions cheat sheet template

Teacher Notes

This session is about language, not clinical technique. Encourage learners to narrate what they normally do physically, translating it into English. Many will find that they know the examination but not the words.

Session 17: Vital Signs – Measurement, Communication & Documentation

CEFR Level B1–B2	Duration 60 minutes	Competency	Vital signs elicitation, interpretation, and reporting
-----------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Correctly communicate all major vital signs in English
- Describe normal and abnormal vital signs using appropriate clinical language
- Interpret vital sign trends and communicate concerns to colleagues
- Document vital signs accurately and in standard clinical format

Outcomes – Learners will demonstrate:

- Can name, measure-describe, and communicate all major vital signs fluently
- Can describe vital sign abnormalities using precise, graded clinical language
- Can report a set of vital signs to a colleague including trends and concerns
- Can document vital signs correctly using standard clinical notation

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Vital signs dictation: teacher reads 8 sets of vital signs aloud in full English; learners transcribe them in numerical format. Identify common transcription errors.
Presentation ~15 min	15 min – Systematic review of each vital sign: name, normal range, unit, and how to read it aloud. Include trend reporting and the language of concern (without alarmism).
Guided Practice ~15 min	15 min – Interpretation activity: pairs receive a vital signs chart showing deterioration over 6 hours. Describe each reading and the overall trend to their partner.
Freer Practice / Production ~15 min	15 min – Telephone handover scenario: doctor calls a colleague to report a deteriorating patient's vital signs. Must communicate all vitals, trends, and recommended action clearly.
Wrap-Up ~5 min	5 min – Review the clinical abbreviations used in documentation. Group generates a 'Vital Signs Quick Reference' card.

Materials Required

1. Vital signs dictation script
2. Vital signs normal range reference card
3. 6-hour vital signs chart

4. Telephone handover scenario cards

5. Quick Reference card template

Teacher Notes

Pay close attention to how learners read blood pressure (e.g., 'one hundred and fifty over ninety' or '150/90'). Both forms are used clinically. Emphasise that communication of vital signs must be unambiguous – lives depend on it.

Session 18: Ordering and Explaining Diagnostic Tests

CEFR Level B2	Duration 60 minutes	Competency	Investigation ordering, explanation, and preparation
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Use appropriate language to order investigations verbally and in writing
- Explain the purpose, procedure, and preparation for common investigations to patients
- Respond to patient questions and concerns about diagnostic tests
- Communicate test results clearly and appropriately to patients and colleagues

Outcomes – Learners will demonstrate:

- Can order common investigations using correct clinical English
- Can explain the purpose and process of a diagnostic test to a patient
- Can answer patient questions about what to expect during a test
- Can communicate whether a test result is normal or abnormal to a patient

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Common investigation flashcard drill: teacher holds up investigation name; learners say what it measures, how it is performed, and when it is indicated. Fast-paced review.
Presentation ~15 min	15 min – Two model explanations: (a) ordering an investigation to a colleague, (b) explaining the test to the patient. Highlight purpose language, preparation instructions, and result framing.
Guided Practice ~15 min	15 min – Match test to purpose, then write a 2-sentence patient explanation for 5 investigations. Peer compare for clarity and accuracy.
Freer Practice / Production ~15 min	15 min – Scenario: patient with chest pain needs ECG, blood tests, and CXR. Doctor must explain all three tests to the patient and answer two concern questions.
Wrap-Up ~5 min	5 min – Exit ticket: write a single-sentence explanation of an MRI for a patient who has never had one.

Materials Required

1. Common investigation flashcards
2. Investigation explanation model texts
3. Matching and explanation worksheet

4. Chest pain scenario roleplay cards

5. Exit ticket slips

Teacher Notes

Many patients are anxious about investigations. Help learners develop language that is reassuring without being dismissive. Distinguish between what you tell a patient and what you document in a clinical order.

Session 19: Interpreting and Presenting Lab Results & Imaging

CEFR Level B2	Duration 60 minutes	Competency	Clinical data interpretation and verbal presentation
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Read and interpret common laboratory and imaging results in English
- Describe findings from blood tests, X-rays, CT, and ECGs using clinical language
- Present results clearly and logically to a colleague or in a clinical setting
- Communicate result implications for diagnosis and management

Outcomes – Learners will demonstrate:

- Can read aloud and interpret a standard blood results panel
- Can describe basic X-ray, ECG, and CT findings using clinical terminology
- Can present investigation results in a structured, logical verbal format
- Can communicate the clinical significance of an abnormal result

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Quick interpretation: show 5 blood results on the board. Learners say each value aloud and flag normal or abnormal. Race format in pairs.

Presentation ~15 min

15 min – Walk through a set of blood results, then a CXR, then an ECG. Model verbal interpretation narration: 'Here we can see... / This suggests... / The key finding is...'

Guided Practice ~15 min

15 min – Results interpretation jigsaw: 4 groups each receive one type of investigation (bloods, CXR, CT, ECG). Prepare a 2-minute verbal report. Regroup to share findings.

Freer Practice / Production ~15 min

15 min – Integrated case: learners receive a patient scenario with all four investigation types. Must give a structured 3-minute verbal presentation of all results and their implications.

Wrap-Up ~5 min

5 min – Discuss: what vocabulary gaps became apparent? Consolidate the 10 most important terms from each investigation type.

Materials Required

1. Blood results panel worksheets
2. CXR and ECG image handouts
3. CT report excerpt

4. Results interpretation jigsaw activity cards

5. Integrated case patient file

Teacher Notes

Imaging and ECG language is very specialist-specific. Focus on common, high-yield findings rather than rarer abnormalities. If possible, use real de-identified clinical images or high-quality clinical simulations.

Session 20: Forming and Communicating a Diagnosis

CEFR Level B2	Duration 60 minutes	Competency	Diagnostic reasoning and patient-facing diagnosis communication
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Use clinical reasoning language to formulate a diagnosis in English
- Present a differential diagnosis list with supporting evidence
- Communicate a diagnosis clearly and sensitively to a patient
- Respond to patient questions about their diagnosis

Outcomes – Learners will demonstrate:

- Can formulate and present a differential diagnosis with ranked clinical reasoning
- Can explain a diagnosis to a patient in lay terms, answering follow-up questions
- Can use appropriately hedged language when diagnosis is uncertain
- Can document a working diagnosis clearly in a clinical note

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Diagnosis probability language: rank 6 expressions from most to least certain (definite, likely, possible, unlikely, ruled out, suspected). Discuss clinical contexts for each.

Presentation ~15 min

15 min – Model a full diagnostic formulation for a case, including differential diagnosis list and patient explanation. Show how clinical reasoning is narrated in English.

Guided Practice ~15 min

15 min – Case formulation task: given patient data, write a 3-sentence diagnostic impression. Present to the class. Peer and teacher feedback on clinical language and confidence calibration.

Freer Practice / Production ~15 min

15 min – Full consultation: doctor must present a diagnosis to a patient, using appropriate language, explaining the basis for the diagnosis, and answering two patient questions.

Wrap-Up ~5 min

5 min – Compile a 'Diagnostic Language Toolkit' – key phrases for presenting diagnoses at various confidence levels.

Materials Required

1. Certainty language ranking cards
2. Diagnostic formulation case handouts

3. Case data sheets
4. Patient roleplay diagnosis cards
5. Diagnostic Language Toolkit template

Teacher Notes

Doctors sometimes default to over-confident or overly vague diagnostic language in a second language. Practise calibrating language to clinical certainty. Use real case studies where available, adjusting for confidentiality.

UNIT 5: TREATMENT & MANAGEMENT

Session 21: Treatment Plans – Explaining Options to Patients

CEFR Level B2	Duration 60 minutes	Competency	Shared decision-making and treatment communication
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Explain treatment options clearly and objectively to a patient in English
- Present the benefits and risks of each treatment option in lay language
- Facilitate shared decision-making by eliciting patient preferences and values
- Summarise an agreed treatment plan clearly

Outcomes – Learners will demonstrate:

- Can present 2–3 treatment options with benefits and risks in accessible language
- Can elicit patient preferences and incorporate them into the treatment discussion
- Can apply the 'Ask-Tell-Ask' shared decision-making technique
- Can summarise and confirm an agreed treatment plan with a patient

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Case vignette: read two descriptions of how a doctor presents a treatment plan – one paternalistic, one shared. Learners identify the differences and what made each feel different.
Presentation ~15 min	15 min – Introduce the Ask-Tell-Ask framework. Model a treatment decision conversation for a patient choosing between surgery and conservative management.
Guided Practice ~15 min	15 min – Sentence builders: fill in treatment option descriptions using provided language frames. Then read aloud to a partner who evaluates clarity.
Freer Practice / Production ~15 min	15 min – Complex scenario: patient must choose between chemotherapy and watchful waiting for a newly diagnosed early-stage cancer. Doctor uses ASK-TELL-ASK to facilitate the discussion.
Wrap-Up ~5 min	5 min – Group reflection: what is the hardest part of shared decision-making language in English? How does this differ from their native language or cultural practice?

Materials Required

1. Two treatment plan vignettes
2. Ask-Tell-Ask framework card
3. Sentence builder worksheet
4. Surgery vs. conservative management scenario pack
5. Cultural reflection prompts

Teacher Notes

Be aware that shared decision-making may be a different cultural norm for some learners. Acknowledge this explicitly and frame it as a professional skill required in English-speaking clinical environments.

Session 22: Pharmacology English – Prescriptions, Dosages & Side Effects

CEFR Level B2	Duration 60 minutes	Competency	Medication communication with patients and colleagues
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Read, write, and communicate prescription information clearly in English
- Explain dosage schedules and administration routes to patients
- Counsel patients on potential side effects and interactions
- Use the language of medication adherence and non-adherence

Outcomes – Learners will demonstrate:

- Can read a prescription aloud accurately, including dose, frequency, and route
- Can explain a medication regimen to a patient in clear lay language
- Can describe and explain the most important side effects of a given medication
- Can address non-adherence in a non-judgmental, supportive manner

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Prescription decoding: learners receive a handwritten prescription and must read it aloud in full sentences. Identifies common difficulties with medical abbreviations (OD, BD, TDS, QDS, prn).

Presentation ~15 min

15 min – Demonstrate a full medication counselling session: read the prescription, explain the drug's purpose, demonstrate dosing instructions, discuss key side effects, address adherence.

Guided Practice ~15 min

15 min – Medication counselling cards: each doctor receives a drug with its key information. They must explain it to a partner in lay language. Partner evaluates clarity.

Freer Practice / Production ~15 min

15 min – Scenario: patient has been started on warfarin. Doctor must explain the drug, why they need it, dosing, INR monitoring, side effects to watch for, and dietary interactions.

Wrap-Up ~5 min

5 min – Class generates a 'Side Effect Language' toolkit: graded language from 'very common' to 'rare but serious'. Review the most misused prescription abbreviations.

Materials Required

1. Handwritten prescription for decoding
2. Medication counselling scenario cards

3. Warfarin explanation scenario
4. Side Effect Language toolkit template
5. Prescription abbreviation reference

Teacher Notes

Medication errors are a leading cause of clinical harm. Emphasise precision and clarity in this session. Use real medication examples wherever possible, and address the serious clinical risks of miscommunication around dosage.

Session 23: Explaining Procedures and Obtaining Consent

CEFR Level B2	Duration 60 minutes	Competency	Procedure explanation and informed consent communication
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Explain a clinical procedure clearly and completely to a patient
- Describe what the patient will experience during the procedure
- Obtain valid informed consent using clear, accessible language
- Respond to patient concerns or refusals with respect and professionalism

Outcomes – Learners will demonstrate:

- Can explain a clinical procedure step-by-step in patient-friendly English
- Can clearly communicate the purpose, process, risks, and benefits of a procedure
- Can obtain informed consent using a structured, legally and ethically sound approach
- Can manage patient refusal or hesitation respectfully and professionally

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Consent scenario vote: present 5 scenarios. Was valid consent obtained? Learners vote and justify. Generates debate about what 'valid' and 'informed' mean in language terms.

Presentation ~15 min

15 min – Model a full procedure explanation and consent conversation for a lumbar puncture. Highlight the 5 elements: purpose, process, benefits, risks, and alternatives (PPBRA).

Guided Practice ~15 min

15 min – Procedure explanation cards: each learner receives a procedure (colonoscopy, pleural tap, biopsy). Write a patient-facing explanation using the PPBRA structure. Share with a partner.

Freer Practice / Production ~15 min

15 min – Full consent roleplay: doctor explains a gastroscopy and obtains consent. Patient is initially hesitant and asks 3 difficult questions. Observer uses a consent checklist.

Wrap-Up ~5 min

5 min – Class discussion: what are the English language risks in consent conversations? (Misunderstood risk statements, false reassurance, etc.) Consolidate language of risk framing.

Materials Required

1. Consent validity discussion cards

2. PPBRA framework handout
3. Procedure explanation cards
4. Gastroscopy scenario pack
5. Consent quality checklist

Teacher Notes

Ensure learners understand that consent is not just a signature – it is a communicative act. Language precision around risk framing is critical. Alert learners to false reassurance phrases (e.g., 'It'll be totally fine, don't worry').

Session 24: Post-Treatment Instructions & Follow-Up Care

CEFR Level B2	Duration 60 minutes	Competency	Patient education and post-treatment communication
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Give clear, structured post-treatment and discharge instructions in English
- Explain what to expect during recovery and when to seek further care
- Check patient comprehension using teach-back techniques
- Arrange follow-up appointments and communicate next steps clearly

Outcomes – Learners will demonstrate:

- Can deliver complete, structured post-treatment instructions verbally
- Can explain warning signs that require immediate medical attention
- Can use the teach-back technique to confirm patient comprehension
- Can communicate a clear follow-up plan including timing and responsible clinician

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – 'What could go wrong?' – learners read a set of vague post-treatment instructions given to a patient. Identify the risks and gaps in clarity. Motivates precision.

Presentation ~15 min

15 min – Model complete post-treatment instructions: structure, sequence, red flags, teach-back, follow-up. Demonstrate the teach-back technique with a volunteer.

Guided Practice ~15 min

15 min – Instruction rewriting: rewrite poorly worded post-treatment instructions for clarity, completeness, and appropriate register.

Freer Practice / Production ~15 min

15 min – Scenario: patient is being discharged after appendectomy. Doctor must deliver all post-operative instructions, check comprehension with teach-back, and arrange follow-up.

Wrap-Up ~5 min

5 min – Why does teach-back matter? Share evidence that it improves outcomes. Class commits to using it in their next English-language consultation.

Materials Required

1. Vague instructions handout
2. Post-treatment instruction model
3. Instruction rewriting worksheet

4. Appendectomy discharge scenario

5. Teach-back observation guide

Teacher Notes

The teach-back technique feels awkward to many doctors at first ('I don't want to make the patient feel stupid'). Frame it as a patient safety tool and a doctor's communication responsibility. Practise the non-condescending phrasing.

Session 25: Emergency Communication – SBAR and Rapid Handover

CEFR Level B2	Duration 60 minutes	Competency	Emergency clinical communication and escalation
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Use the SBAR framework to communicate clinical urgency clearly
- Give a rapid, structured verbal handover in an emergency setting
- Use language that communicates urgency without causing panic
- Request urgent help clearly and directly in clinical English

Outcomes – Learners will demonstrate:

- Can apply the SBAR framework to a deteriorating patient communication
- Can give a complete, structured emergency handover in under 90 seconds
- Can communicate clinical urgency with appropriate directness and precision
- Can ask for urgent help using assertive, professional escalation language

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Audio clip: two emergency handovers (one unclear, one using SBAR). Learners identify what made each effective or ineffective.
Presentation ~15 min	15 min – Teach the SBAR framework. Model a deteriorating patient handover using SBAR. Emphasise language of urgency, directness, and the clinical recommendation.
Guided Practice ~15 min	15 min – SBAR card-building exercise: given a patient scenario, construct each SBAR element in bullet points. Present to the group as a verbal handover. Time it.
Freer Practice / Production ~15 min	15 min – High-stakes roleplay: septic patient deteriorating on the ward. Junior doctor must call the on-call senior using SBAR to escalate clearly and effectively.
Wrap-Up ~5 min	5 min – Group discuss: 'What stops people from being direct in emergency English?' Address cultural hesitation, hierarchy concerns, and language confidence.

Materials Required

1. Emergency handover audio clips
2. SBAR framework card
3. SBAR construction activity cards

4. Septic patient scenario
5. Time-your-handover stopwatch exercise

Teacher Notes

This session often reveals hierarchical hesitation – doctors may be reluctant to give clear recommendations to seniors. Validate this and practise assertive but respectful escalation language. Psychological safety in communication saves lives.

UNIT 6: CLINICAL DOCUMENTATION & WRITING

Session 26: Writing SOAP Notes in English

CEFR Level B1–B2	Duration 60 minutes	Competency	Clinical documentation – SOAP format
-----------------------------------	-------------------------------	-------------------	--------------------------------------

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Write clear, structured SOAP notes in English using appropriate clinical language
- Distinguish between subjective, objective, assessment, and plan documentation
- Use standard clinical abbreviations and terminology correctly in written notes
- Review and improve the clarity and completeness of a clinical note

Outcomes – Learners will demonstrate:

- Can write a complete SOAP note in English for a clinical encounter
- Can correctly categorise clinical information into S, O, A, and P sections
- Can use standard abbreviations correctly in written documentation
- Can identify and correct errors in a poorly written clinical note

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – SOAP sorting: learners receive 20 clinical information snippets and sort them into S, O, A, P categories. Review and discuss any disagreements.
Presentation ~15 min	15 min – Walk through a full SOAP note for a complete case. Highlight language conventions: passive voice, abbreviations, tense use, and the difference between documenting patient-reported symptoms vs. clinical findings.
Guided Practice ~15 min	15 min – Write the S and O sections for a provided clinical case. Exchange with a partner for feedback on clarity, language, and completeness.
Freer Practice / Production ~15 min	15 min – Complete SOAP note writing task: learners receive a consultation transcript and must write the full note from scratch. Teacher provides structured feedback.
Wrap-Up ~5 min	5 min – Common SOAP note errors review: illegible abbreviations, mixing S and O information, vague plans. Class agrees on a 5-point quality checklist.

Materials Required

1. SOAP sorting activity cards

2. Model SOAP note example
3. Clinical case for S&O practice
4. Consultation transcript for full note writing
5. 5-point quality checklist template

Teacher Notes

SOAP documentation varies by institution. Acknowledge this variation but focus on the universal principles. Emphasise that documentation is a medicolegal document and precision in language is non-negotiable.

Session 27: Clinical Summaries and Case Reports

CEFR Level B2	Duration 60 minutes	Competency	Professional medical writing – summaries and case reports
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Write a structured, concise clinical summary of a patient case
- Use passive constructions and formal tense conventions in medical writing
- Organise a clinical case report following standard academic structure
- Write clearly for a professional medical audience

Outcomes – Learners will demonstrate:

- Can produce a clear, logically structured clinical case summary
- Can use formal written medical English with appropriate passive constructions and tense
- Can organise a case report using: introduction, case presentation, discussion, conclusion
- Can identify and self-correct common errors in formal medical writing

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Rewrite the summary: learners receive a poorly written clinical summary (overly informal, disorganised, incorrect tense). Identify all errors before rewriting begins.
Presentation ~15 min	15 min – Model a well-written clinical summary and a case report structure. Highlight passive voice conventions, formal transition language, and clinical hedging.
Guided Practice ~15 min	15 min – Transform an active-voice clinical text into a formal passive summary. Compare with partner. Discuss whether passive is always appropriate.
Freer Practice / Production ~15 min	15 min – Write a 150-word clinical summary from a complex provided case. Focus on selection, organisation, and formal language.
Wrap-Up ~5 min	5 min – Peer review using a 4-criterion rubric (accuracy, organisation, register, conciseness). Set target word counts and revision strategies.

Materials Required

1. Poorly written summary for error identification
2. Model clinical summary and case report

3. Active-to-passive transformation exercise
4. Complex case for summary writing
5. 4-criterion peer review rubric

Teacher Notes

Medical writing in English uses formal conventions that may differ from other languages or medical traditions. Focus on: present perfect for ongoing conditions, past simple for acute events, and passive voice for clinical procedures.

Session 28: Referral Letters and Medical Correspondence

CEFR Level B2	Duration 60 minutes	Competency	Professional written communication – referrals and letters
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Write a structured, professional referral letter in English
- Include all essential clinical information in the correct order
- Use appropriate formal language for different types of medical correspondence
- Write professionally to GPs, specialists, and other healthcare providers

Outcomes – Learners will demonstrate:

- Can write a complete, well-structured referral letter including all essential components
- Can select and present relevant clinical information efficiently in written form
- Can adapt tone and content for different types of recipient (GP, specialist, insurer)
- Can identify and correct errors of content, structure, and language in a referral letter

Lesson Plan & Teaching Guide

Warm-Up

~10 min

10 min – Annotate the letter: learners identify which sections of a referral letter are missing or unclear. Motivates understanding of the genre structure.

Presentation

~15 min

15 min – Present the standard referral letter structure: header, opening, reason, background, current management, request, closing. Highlight formal register features.

Guided Practice

~15 min

15 min – Referral letter framework fill-in: complete a referral letter for a provided case using the structure and language bank. Compare with a partner.

Freer Practice / Production

~15 min

15 min – Write a referral letter from scratch for a case involving a new diagnosis and required specialist input. Peer review using a structure checklist.

Wrap-Up

~5 min

5 min – Compile a 'Referral Letter Phrase Bank' – class contributes their strongest sentences from the task.

Materials Required

1. Annotated incomplete referral letter
2. Referral letter structure guide

3. Case and language bank for fill-in task
4. New diagnosis case for full letter
5. Structure checklist
6. Phrase bank template

Teacher Notes

Referral letters in English follow fairly standardised conventions. Emphasise conciseness – referral letters should be comprehensive but not exhaustive. Common error: including irrelevant history or omitting the specific request.

Session 29: Discharge Summaries

CEFR Level B2	Duration 60 minutes	Competency	Professional medical writing – discharge documentation
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Write a complete, structured discharge summary in English
- Accurately document the admission diagnosis, treatment, and outcomes
- Clearly communicate ongoing management needs to the receiving clinician
- Use concise, precise clinical language in discharge documentation

Outcomes – Learners will demonstrate:

- Can write a complete discharge summary covering all essential components
- Can document the admission, investigations, treatment, and follow-up plan clearly
- Can write for a GP audience: complete but accessible
- Can identify gaps or errors in a given discharge summary

Lesson Plan & Teaching Guide

Warm-Up

~10 min

10 min – Discharge summary detective: given a discharge summary with 6 errors or omissions, find them all. Teams compete. Establishes quality criteria.

Presentation

~15 min

15 min – Model a complete discharge summary with live annotation. Emphasise the recipient (GP) and what they need to know to continue care safely.

Guided Practice

~15 min

15 min – Section-by-section writing: learners write the investigations and treatment sections of a provided case. Exchange for feedback.

Freer Practice / Production

~15 min

15 min – Full discharge summary from a complex case (e.g., MI with complications). Write independently; teacher circulates and provides formative feedback.

Wrap-Up

~5 min

5 min – Highlight the top 3 documentation errors that cause adverse events post-discharge (missing medication changes, unclear follow-up, unreported allergies). Emphasise real clinical impact.

Materials Required

1. Discharge summary with 6 errors
2. Model annotated discharge summary

3. Section-writing activity case
4. MI complex case for full writing
5. Formative feedback rubric

Teacher Notes

Discharge summaries are often written under time pressure. Efficiency of language is as important as completeness. Discuss the risks of copy-paste documentation and the importance of specificity in medication and follow-up instructions.

Session 30: Medical Terminology – Prefixes, Suffixes & Word Building

CEFR Level B1–B2	Duration 60 minutes	Competency	Medical word structure and vocabulary expansion
-----------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Identify and apply common medical prefixes, suffixes, and root words in English
- Deduce the meaning of unfamiliar medical terms using word structure
- Build and correctly use medical compound terms
- Distinguish between British and American English spelling variants in medical terms

Outcomes – Learners will demonstrate:

- Can correctly apply 20 high-frequency medical prefixes and suffixes
- Can decode unfamiliar medical terms using structural analysis
- Can construct accurate medical compound terms from given components
- Can identify common British/American spelling differences (e.g., haemoglobin/hemoglobin)

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Word decoding race: teams receive 10 unfamiliar medical terms and must write their meaning using structural analysis only. No dictionaries. Reveals natural word-building intuition.
Presentation ~15 min	15 min – Systematic review of high-frequency prefixes and suffixes. Provide a word-building reference chart. Demonstrate how combining forms create meaning.
Guided Practice ~15 min	15 min – Word-building activity: given root + prefix + suffix, construct a medical term and write its definition. Then do the reverse: given definition, construct the term.
Freer Practice / Production ~15 min	15 min – Medical terminology quiz game (teams): teacher reads a definition; teams construct the medical term. Bonus points for correct British AND American spellings.
Wrap-Up ~5 min	5 min – Class builds a 'Master Terminology Reference' document collaboratively. Each doctor contributes 3 terms they use regularly in their specialty.

Materials Required

1. Word decoding race cards
2. Prefix/Suffix/Root reference chart

3. Word-building worksheet
4. Terminology quiz game cards
5. Master Terminology Reference template

Teacher Notes

Morphological awareness transfers strongly to real clinical reading. Doctors who can decode new terms independently become more confident with medical literature. This session builds that metacognitive vocabulary skill.

UNIT 7: PROFESSIONAL & INTERDISCIPLINARY COMMUNICATION

Session 31: Communicating with Colleagues – Handovers & Briefings

CEFR Level B2	Duration 60 minutes	Competency	Interprofessional clinical communication
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Give a clear, complete, and efficient clinical handover to a colleague
- Participate in and lead a clinical briefing in English
- Use professional language when communicating clinical concerns to colleagues
- Manage interruptions, clarifications, and follow-up questions in a clinical handover

Outcomes – Learners will demonstrate:

- Can give a structured handover covering all essential patient information
- Can lead and contribute to a clinical briefing using appropriate professional language
- Can manage clarifying questions and follow-up during a verbal handover
- Can use the I-PASS handover tool fluently in English

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Handover audit: listen to a 2-minute simulated handover. Identify all the missing pieces of information using a standardised checklist.
Presentation ~15 min	15 min – Introduce the I-PASS handover tool. Model a complete I-PASS handover for a complex patient. Highlight language for flagging concerns and action items.
Guided Practice ~15 min	15 min – I-PASS card-building: groups receive patient information in random order. Sort it into I-PASS categories, then deliver a 90-second handover.
Freer Practice / Production ~15 min	15 min – Simulation: end-of-shift handover for a 5-patient ward. Teams hand over patients to each other. Receiving team asks at least 2 clarifying questions.
Wrap-Up ~5 min	5 min – Class identifies the three elements of a handover most commonly dropped in English. Consolidate the language of 'flagging' clinical concerns.

Materials Required

1. Simulated handover audio (or transcript)

2. I-PASS framework card
3. I-PASS card-sorting packs
4. 5-patient ward handover scenario
5. Clarifying question prompts

Teacher Notes

Handover communication failures are a leading cause of adverse events. Reinforce the ethical and safety dimensions of this session. Encourage learners to use I-PASS in their own settings.

Session 32: Case Presentations in English

CEFR Level B2	Duration 60 minutes	Competency	Formal oral case presentation skills
--------------------------------	-------------------------------	-------------------	--------------------------------------

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Structure and deliver a formal case presentation in English
- Use appropriate clinical language and organisation for a professional audience
- Manage questions and discussion following a case presentation
- Adapt a case presentation to different contexts (teaching round, grand round, case conference)

Outcomes – Learners will demonstrate:

- Can deliver a structured, fluent 5-minute case presentation in English
- Can use the standard case presentation sequence (ID, CC, HPC, PMH, SH, FH, E/O, IX, A, P)
- Can respond to clarifying or challenging questions during a case discussion
- Can adjust the level of detail in a presentation based on the audience and context

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Watch a 3-minute case presentation clip (real or simulated). Learners score it on 4 criteria: structure, clarity, language accuracy, and engagement. Discuss.

Presentation ~15 min

15 min – Map out the full case presentation sequence on the board. Demonstrate how to move efficiently between sections. Show how to hedge and present uncertainty.

Guided Practice ~15 min

15 min – Strip story: cut up a well-written case presentation into strips. Teams re-order it correctly, then identify the transition language used.

Freer Practice / Production ~15 min

15 min – Each learner delivers a 3-minute case presentation (using a provided or own case). Group listens and asks 1–2 questions. Observer uses a structure checklist.

Wrap-Up ~5 min

5 min – Class reflection: what aspects of case presentation are most difficult in English? Discuss pacing, vocabulary retrieval, and question handling.

Materials Required

1. Case presentation video clip
2. Case presentation sequence guide

3. Strip story activity set
4. Presentation cases or prompts for own case
5. Observer structure checklist

Teacher Notes

Case presentations are a key professional gatekeeping skill. Many non-native English doctors feel this is where their language barrier is most visible. Practise extensively and provide positive specific feedback to build confidence.

Session 33: Multidisciplinary Team (MDT) Meetings

CEFR Level B2	Duration 60 minutes	Competency	Collaborative decision-making in team settings
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Participate actively and appropriately in a simulated MDT meeting in English
- Present a patient case concisely for MDT discussion
- Contribute opinions and recommendations using professional language
- Manage disagreement, clarification, and consensus-building in a team setting

Outcomes – Learners will demonstrate:

- Can present a patient case concisely and clearly in an MDT context
- Can contribute professional opinions and recommendations during team discussion
- Can agree, politely disagree, and seek clarification in a team meeting
- Can summarise an MDT decision and document agreed actions

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – MDT role assignment: learners are assigned roles (oncologist, surgeon, radiologist, palliative care nurse, GP). Quick vocabulary review of each role's scope and typical contributions.
Presentation ~15 min	15 min – Show a short video or transcript of a well-run MDT. Discuss: what language made contributions effective? How was disagreement handled? What made the summary clear?
Guided Practice ~15 min	15 min – Language drills: agree/disagree/qualify sentence transformations. Then practise presenting a case in under 90 seconds using an MDT case summary card.
Freer Practice / Production ~15 min	15 min – Simulated MDT meeting: facilitator (teacher or learner) runs the meeting. Each role-holder must contribute once, respond to one other, and the group must reach a decision.
Wrap-Up ~5 min	5 min – Debrief: what language worked well? Who dominated? Who was not heard? How can you ensure your voice is heard in a real MDT?

Materials Required

1. MDT role cards
2. MDT video clip or transcript
3. Agree/disagree/qualify drill cards

4. MDT case summary cards
5. Simulated MDT patient pack
6. Observer feedback sheet

Teacher Notes

MDT dynamics are complex. Non-native speakers often hold back in MDTs due to linguistic confidence. Create a psychologically safe simulation environment. Explicitly coach assertive but respectful contribution.

Session 34: Telehealth & Remote Patient Communication

CEFR Level B2	Duration 60 minutes	Competency	Digital and remote consultation skills
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Conduct effective patient consultations via telephone and video in English
- Manage the communicative challenges specific to remote consultations
- Check patient comprehension and establish non-verbal connection remotely
- Document a telehealth consultation appropriately

Outcomes – Learners will demonstrate:

- Can open, conduct, and close a telephone or video consultation professionally
- Can compensate for the absence of non-verbal cues using verbal strategies
- Can manage technical difficulties, silence, and misunderstanding in remote consultations
- Can document a remote consultation including the modality used and any limitations

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – What's different? Learners brainstorm the key differences between in-person and telehealth consultations. Compile two lists: what's lost and what's gained.
Presentation ~15 min	15 min – Model a telephone consultation focusing on: clear identification, more explicit verbal cues, more checking of comprehension, structured closing with safety netting.
Guided Practice ~15 min	15 min – Back-to-back telephone roleplay: participants sit back-to-back (or use phones). Doctor consults patient; no visual contact allowed. Reflect on the difference.
Freer Practice / Production ~15 min	15 min – Video consultation simulation (if technology available) or telephone roleplay for a complex case requiring safety netting. Observer rates on a telehealth-specific checklist.
Wrap-Up ~5 min	5 min – Discuss: what special language strategies are essential for remote consultations? When is in-person a necessity?

Materials Required

1. In-person vs. telehealth comparison activity sheet
2. Telephone consultation model script

3. Roleplay scenario cards (back-to-back setup)
4. Telehealth-specific observer checklist
5. Safety netting language card

Teacher Notes

Telehealth has grown significantly. Many of the learners' patients may prefer or require it. Emphasise the ethical responsibility of knowing when remote consultation is insufficient and in-person referral is needed.

Session 35: Reading & Discussing Medical Literature

CEFR Level B2	Duration 60 minutes	Competency	Academic English and evidence-based medicine
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Read and comprehend a medical research abstract in English
- Identify and discuss key components of a research article (method, findings, limitations)
- Critically evaluate evidence quality using appropriate academic language
- Discuss clinical implications of a study using evidence-based language

Outcomes – Learners will demonstrate:

- Can identify and summarise the key components of a medical research abstract
- Can discuss research findings using appropriate academic hedging
- Can critically evaluate research quality using relevant criteria
- Can apply study findings to a clinical scenario and justify using evidence-based language

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Abstract treasure hunt: learners receive a 1-page abstract and must find: the study design, sample size, primary outcome, and main finding. Timed. Builds scanning skills.
Presentation ~15 min	15 min – Walk through a model article, section by section. Identify language conventions for each section (methods use passive past, discussion uses hedged present). Teach critical reading questions.
Guided Practice ~15 min	15 min – Group jigsaw: 4 groups each read one section of an article (intro, methods, results, discussion). Regroup to present their section and reconstruct the full study.
Freer Practice / Production ~15 min	15 min – Journal club simulation: learners present a brief (3-minute) critique of a study and its clinical implications. Others ask one critical question each.
Wrap-Up ~5 min	5 min – Build a 'Critical Appraisal Language Toolkit' together: phrases for evaluating strength of evidence, study limitations, and clinical relevance.

Materials Required

1. Medical research abstract for treasure hunt
2. Full-article handout (appropriate length)

3. Section-reading jigsaw assignment cards
4. Journal club presentation guide
5. Critical Appraisal Language Toolkit template

Teacher Notes

Many doctors can read English medical literature but struggle to talk about it. This session specifically develops oral academic discussion. The goal is fluency in evidence-based dialogue, not mastery of statistics.

UNIT 8: COMPLEX & SENSITIVE COMMUNICATION

Session 36: Breaking Bad News – The SPIKES Protocol

CEFR Level B2	Duration 60 minutes	Competency	Sensitive communication and emotional support in critical conversations
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Apply the SPIKES protocol to deliver serious news sensitively in English
- Use language that is honest, compassionate, and clearly understood by the patient
- Respond to emotional reactions (shock, anger, denial, grief) during a bad news consultation
- Provide appropriate support and next-step information after delivering bad news

Outcomes – Learners will demonstrate:

- Can conduct a bad news consultation using the SPIKES framework
- Can use language that is honest yet compassionate and appropriately paced
- Can respond to a range of emotional reactions with empathy and professional composure
- Can provide a clear, manageable plan and emotional support following bad news

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Watch a 3-minute video of a bad news consultation (good or bad example). Learners identify what helped or hurt. Surfaces existing instincts before formal teaching.
Presentation ~15 min	15 min – Introduce SPIKES. Live demonstration of a complete bad news conversation (cancer diagnosis). Narrate each SPIKES stage as it happens. Highlight specific language choices.
Guided Practice ~15 min	15 min – SPIKES language sorting: 20 phrases sorted into the 6 SPIKES stages. Discuss any ambiguous cases. Then rewrite 5 blunt or inappropriate statements using SPIKES language.
Freer Practice / Production ~15 min	15 min – Highly emotionally realistic roleplay: patient receives a serious diagnosis (e.g., terminal cancer, MS, HIV). Doctor must use SPIKES and navigate one unexpected emotional reaction.
Wrap-Up ~5 min	5 min – Debrief with honesty: How did that feel? What was hardest to say in English? What words helped? This session often evokes strong responses – honour them.

Materials Required

1. Bad news video clip
2. SPIKES framework handout
3. SPIKES language sorting cards
4. Rewriting blunt statements worksheet
5. Realistic diagnosis roleplay scenario packs
6. Emotional debrief guide

Teacher Notes

This is the most emotionally intensive session in the curriculum. Brief learners before starting. Debrief thoroughly after. Some learners may connect this to personal experiences – be prepared to support this. Never rush the debrief.

Session 37: Informed Consent, Patient Rights & Shared Decision-Making

CEFR Level B2	Duration 60 minutes	Competency	Ethical communication and patient autonomy
--------------------------------	-------------------------------	-------------------	--

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Explain patient rights using clear, accessible English
- Conduct a complete informed consent process for a complex procedure
- Support patient autonomy and independent decision-making through language choices
- Respond to refusal of treatment respectfully and ethically

Outcomes – Learners will demonstrate:

- Can explain the legal and ethical basis for informed consent in clear language
- Can conduct a complete, valid consent process for a complex medical scenario
- Can use language that respects and promotes patient autonomy
- Can manage and document patient refusal professionally and ethically

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Ethical scenarios vote: 5 clinical scenarios. Was consent valid? Learners vote yes/no and justify. Raises debate about capacity, coercion, and what 'informed' really means.

Presentation ~15 min

15 min – Review elements of valid consent (voluntary, informed, capacity). Demonstrate a consent conversation that models genuine choice. Contrast with a paternalistic alternative.

Guided Practice ~15 min

15 min – Consent language transformation: rewrite paternalistic or coercive consent language into autonomy-respecting language. Discuss the impact of each version.

Freer Practice / Production ~15 min

15 min – Complex consent scenario: patient refuses a blood transfusion for religious reasons. Doctor must navigate the conversation, document the refusal, and ensure the patient's decision is truly voluntary.

Wrap-Up ~5 min

5 min – Class discussion: How does English language proficiency affect a patient's ability to give valid consent? What are our responsibilities as their doctor?

Materials Required

1. Ethical scenarios cards
2. Elements of consent reference card

3. Consent language transformation worksheet

4. Blood transfusion refusal scenario

5. Consent documentation template

Teacher Notes

This session touches deeply on medical ethics. Allow genuine discussion. Connect the language to real ethical obligation: a patient who doesn't fully understand what they're consenting to has not given valid consent.

Session 38: Cross-Cultural Communication in Medicine

CEFR Level B2	Duration 60 minutes	Competency	Cultural sensitivity and effective intercultural healthcare communication
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Identify potential cultural differences that affect medical communication in English
- Adapt communication style to respect patients' cultural backgrounds
- Use culturally sensitive language when discussing sensitive topics
- Navigate language barriers and work effectively with interpreters

Outcomes – Learners will demonstrate:

- Can identify key cultural dimensions that affect clinical communication
- Can adapt history-taking and explanation language for cultural sensitivity
- Can work effectively with a professional medical interpreter
- Can avoid cultural assumptions and apply patient-centred, individuality-respecting language

Lesson Plan & Teaching Guide

Warm-Up ~10 min

10 min – Cultural communication quiz: 10 statements about clinical communication norms. True or false? Discuss. Examples: 'Eye contact is always a sign of honesty' / 'Nodding means agreement'. Challenges assumptions.

Presentation ~15 min

15 min – Explore key cultural dimensions in healthcare: directness, hierarchy, illness beliefs, family involvement, gender norms, and disclosure practices. Use real clinical examples.

Guided Practice ~15 min

15 min – Case studies: 3 brief scenarios involving cultural misunderstandings. Groups analyse what happened, what assumptions were made, and how language could have been adapted.

Freer Practice / Production ~15 min

15 min – Roleplay with interpreter: 3-way simulation (doctor, interpreter, patient). Doctor must speak in short segments, allow for interpretation, and maintain connection with the patient.

Wrap-Up ~5 min

5 min – Class reflection: what cultural assumptions do we carry into our consultations? How does being a non-native English speaker give you insight into cross-cultural communication?

Materials Required

1. Cultural communication quiz cards

2. Cultural dimensions reference sheet
3. Three-way roleplay scenario pack
4. Interpreter briefing card
5. Cultural reflection discussion prompts

Teacher Notes

This session should be handled with curiosity, not prescriptiveness. Avoid stereotyping. Emphasise that all patients are individuals. The goal is cultural humility, not cultural checklists.

Session 39: Ethics, Confidentiality & Professional Boundaries in English

CEFR Level B2	Duration 60 minutes	Competency	Professional and ethical communication in English
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Discuss key medical ethics principles (autonomy, beneficence, non-maleficence, justice) in English
- Explain confidentiality obligations and exceptions to patients and colleagues
- Use appropriate professional language when addressing ethical concerns
- Navigate professional boundary discussions using respectful, clear English

Outcomes – Learners will demonstrate:

- Can explain the 4 principles of medical ethics in accessible language
- Can clearly communicate confidentiality policies to patients
- Can discuss an ethical dilemma professionally using structured reasoning language
- Can address professional boundary issues using firm but respectful English

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – Ethics dilemma card sort: 5 clinical dilemmas. Groups rank them from 'easiest' to 'hardest' ethical decision. Debate and justify. No right answers – generates discussion.
Presentation ~15 min	15 min – Review the 4 principles using real clinical examples. Then present confidentiality rules in English (GDPR language, duty of care exceptions). Model professional boundary language.
Guided Practice ~15 min	15 min – Ethics discussion: in groups of 3, discuss a complex ethical scenario (e.g., a patient asks you not to tell their family about a terminal diagnosis). Use structured debate language.
Freer Practice / Production ~15 min	15 min – Roleplay: patient challenges the doctor to break confidentiality for their family member, or makes a request that crosses a professional boundary. Doctor responds professionally.
Wrap-Up ~5 min	5 min – Summary: what language made the ethical position clear while remaining empathic? Consolidate 5 professional boundary phrases.

Materials Required

1. Ethics dilemma card sort
2. 4 principles reference card with examples

3. Confidentiality policy summary card
4. Complex ethics scenario discussion card
5. Professional boundary phrasebank

Teacher Notes

Some ethical frameworks vary between healthcare systems. Acknowledge this and focus on universally applicable communication strategies. This session prepares learners for real ethical conversations in English-speaking healthcare environments.

Session 40: Integrated Review, Reflection & Mock Consultation Assessment

CEFR Level B2	Duration 60 minutes	Competency	Integrated medical English communication – all competencies
--------------------------------	-------------------------------	-------------------	---

Session Objectives & Outcomes

Objectives – Learners will be able to:

- Demonstrate integrated use of all major consultation and communication skills from the course
- Self-reflect on learning progress and identify remaining language development goals
- Perform a full simulated clinical consultation in English for assessment
- Give and receive structured, constructive feedback on clinical communication performance

Outcomes – Learners will demonstrate:

- Can conduct a complete, integrated clinical consultation in English
- Can identify personal strengths and areas for further development in medical English
- Can give specific, evidence-based feedback to a peer on their consultation performance
- Can set a personal post-course language development action plan

Lesson Plan & Teaching Guide

Warm-Up ~10 min	10 min – 'Course in a minute': each learner shares the one most important thing they learned from the course. Quick gallery of answers on the board. Celebrates learning.
Presentation ~15 min	10 min – Brief review of the full consultation framework (all skills covered). Introduce the mock consultation assessment format: 10-minute full consultation with peer observer and teacher.
Guided Practice ~15 min	10 min – Final warm-up roleplay: 2-minute consultation practice with a simple scenario. Focus on any last-minute language confidence building.
Freer Practice / Production ~15 min	25 min – Mock consultations: each learner conducts a 10-minute full consultation. Teacher and peer use the comprehensive assessment checklist. All skills assessed. Learners rotate roles.
Wrap-Up ~5 min	5 min – Individual reflection time: complete the course learning reflection worksheet and personal action plan. Share one goal with the group. Teacher offers encouragement and individual notes.

Materials Required

1. Mock consultation scenario packs
2. Comprehensive assessment checklist (from Part 4)
3. Peer feedback forms
4. Course learning reflection worksheet
5. Personal action plan template

Teacher Notes

This final session is celebratory as well as evaluative. Balance rigorous assessment with recognition of achievement. Every learner has made significant progress. Provide written individual feedback notes if possible – they are highly valued.

PART FOUR: TEACHER ASSESSMENT CHECKLIST

Author: Rayene Medabis

This section provides a comprehensive set of assessment tools for teachers to evaluate the success of the Medical English course. The checklists cover learner performance, teacher delivery, course effectiveness, and session-by-session monitoring. Assessment is designed to be formative (ongoing) as well as summative (end-of-course).

CHECKLIST 1 – Learner Oral Communication Performance

Use this checklist during or after oral tasks, roleplays, and mock consultations. Rate each criterion on the scale: 1 = Beginning | 2 = Developing | 3 = Competent | 4 = Proficient

A. VOCABULARY & LANGUAGE ACCURACY		
Medical vocabulary range	Uses a wide range of appropriate medical terms	1 2 3 4
Vocabulary precision	Selects vocabulary that is clinically accurate	1 2 3 4
Register appropriateness	Switches between formal and patient-friendly language	1 2 3 4
Grammar accuracy	Produces grammatically correct sentences in clinical speech	1 2 3 4
Pronunciation clarity	Pronounces medical terms clearly and accurately	1 2 3 4
B. CONSULTATION SKILLS		
Opening the consultation	Establishes rapport and greets the patient professionally	1 2 3 4
History taking	Covers all relevant areas systematically and sensitively	1 2 3 4
Questioning technique	Balances open, closed, and probing questions appropriately	1 2 3 4
Active listening	Uses listener responses, reflection, and paraphrasing	1 2 3 4
Empathic communication	Responds to emotions with appropriate empathy language	1 2 3 4
Patient explanation	Explains diagnosis, treatment, and instructions clearly	1 2 3 4
Closing the consultation	Summarises, confirms understanding, and arranges follow-up	1 2 3 4

C. DOCUMENTATION & WRITING

SOAP note accuracy	Writes complete, accurate, well-structured SOAP notes	1 2 3 4
Referral/discharge writing	Produces professional, complete written communications	1 2 3 4
Medical terminology usage	Uses prefixes, suffixes, and clinical terms correctly in writing	1 2 3 4
Formal register in writing	Maintains formal, professional tone in clinical documents	1 2 3 4

D. PROFESSIONAL COMMUNICATION

Case presentation skills	Delivers structured, fluent case presentations	1 2 3 4
Handover communication	Gives clear, complete, and safe verbal handovers	1 2 3 4
MDT participation	Contributes effectively and appropriately in team discussions	1 2 3 4
Sensitive communication	Handles emotionally complex conversations professionally	1 2 3 4

E. OVERALL COMMUNICATION EFFECTIVENESS

Fluency	Speaks with sufficient fluency for clinical contexts	1 2 3 4
Confidence	Communicates with self-assurance and professional presence	1 2 3 4
Adaptability	Adjusts communication for patient, context, and purpose	1 2 3 4
Safety communication	Communicates clinical information clearly and safely	1 2 3 4

Overall Oral Communication Score: _____ / 100

Comments & Areas for Development:

CHECKLIST 2 – Learner Written Communication Performance

Use this checklist to assess written work including SOAP notes, referral letters, discharge summaries, and case summaries.

A. CONTENT & COMPLETENESS		
Clinical accuracy	All clinical information is medically accurate and complete	1 2 3 4
Relevant information	Includes only clinically relevant details; avoids unnecessary padding	1 2 3 4
Key information present	Includes diagnosis, management, and follow-up appropriately	1 2 3 4
B. STRUCTURE & ORGANISATION		
Correct document format	Follows the expected format (SOAP, referral, discharge)	1 2 3 4
Logical sequencing	Information is presented in a logical, readable order	1 2 3 4
Appropriate sections	Uses correct headings and sectioning for the document type	1 2 3 4
C. LANGUAGE & STYLE		
Formal register	Maintains formal, professional written language throughout	1 2 3 4
Medical vocabulary	Uses correct and appropriate medical terminology	1 2 3 4
Grammar & syntax	Sentences are grammatically correct and clear	1 2 3 4
Conciseness	Achieves completeness without unnecessary repetition or wordiness	1 2 3 4
Abbreviations	Uses standard clinical abbreviations correctly	1 2 3 4
D. CLINICAL COMMUNICATION		
Clarity for recipient	Written communication would be clear to the intended recipient	1 2 3 4
Action orientation	Plan and follow-up instructions are clear and actionable	1 2 3 4
Patient-centred language	Where appropriate, communication is accessible and respectful	1 2 3 4

Overall Written Communication Score: _____ / 60

Comments & Areas for Development:

CHECKLIST 3 – Course Delivery & Teaching Quality

For teacher self-evaluation or peer observation of teaching quality. Rate each criterion: 1 = Needs Improvement | 2 = Satisfactory | 3 = Good | 4 = Excellent

A. LESSON PLANNING & STRUCTURE		
Objectives were clear	Lesson objectives were stated and achieved	1 2 3 4
Activities were appropriate	Tasks were suitably challenging for the B1/B2 level	1 2 3 4
Timing was managed	Each lesson phase was completed within the allocated time	1 2 3 4
Materials were effective	Handouts and resources supported learning appropriately	1 2 3 4
B. TEACHING METHODOLOGY		
Authentic medical context	All language was taught in realistic clinical contexts	1 2 3 4
Communicative approach	Learners spent majority of time practising communication	1 2 3 4
Balanced skills coverage	Speaking, listening, reading, and writing were all addressed	1 2 3 4
Scaffolding was effective	Support was provided before independent production tasks	1 2 3 4
Register and vocabulary	Medical vocabulary was taught with appropriate context	1 2 3 4
C. LEARNER ENGAGEMENT		
Learner participation	All doctors were actively engaged throughout the session	1 2 3 4
Inclusive environment	All learners felt safe to practise and make errors	1 2 3 4
Medical knowledge respected	Doctors' clinical expertise was recognised and utilised	1 2 3 4
Feedback quality	Error correction was timely, constructive, and accurate	1 2 3 4
D. COURSE MANAGEMENT		
Curriculum alignment	Sessions followed the planned curriculum sequence	1 2 3 4

A. LESSON PLANNING & STRUCTURE

Formative assessment	Teacher monitored and recorded learner progress regularly	1 2 3 4
Adaptation to learner needs	Lessons were adjusted based on observed learner difficulties	1 2 3 4
Professional atmosphere	Tone was professional, respectful, and motivating	1 2 3 4

Overall Teaching Quality Score: _____ / 72

Reflections & Development Actions:

CHECKLIST 4 – Session-by-Session Progress Monitoring

Complete after each session to track learner engagement, performance, and teacher observations.

Session	Topic	Attendance	Engagement	Objective Met?	Key Observations
1	Introduction to Medical English & Professional I...	—	1 2 3 4	Y / N	
2	The Human Body – Anatomy Vocabulary in Context	—	1 2 3 4	Y / N	
3	Medical Specialties, Roles & Healthcare Settings	—	1 2 3 4	Y / N	
4	Numbers, Measurements & Medical Data	—	1 2 3 4	Y / N	
5	Professional Register – Formal vs. Informal Lang...	—	1 2 3 4	Y / N	
6	Opening Consultations & Establishing Rapport	—	1 2 3 4	Y / N	
7	Eliciting the Chief Complaint	—	1 2 3 4	Y / N	
8	Describing and Asking About Symptoms	—	1 2 3 4	Y / N	
9	Exploring Pain – The SOCRATES Framework	—	1 2 3 4	Y / N	
10	Active Listening & Empathic Language	—	1 2 3 4	Y / N	
11	Past Medical History, Medications & Allergies	—	1 2 3 4	Y / N	
12	Family History & Genetic Risk Communication	—	1 2 3 4	Y / N	
13	Social History & Lifestyle Factors	—	1 2 3 4	Y / N	
14	Review of Systems in English	—	1 2 3 4	Y / N	
15	Pediatric & Geriatric Communication Adaptations	—	1 2 3 4	Y / N	
16	The Physical Examination – Instructions & Vocabu...	—	1 2 3 4	Y / N	

Session	Topic	Attendance	Engagement	Objective Met?	Key Observations
17	Vital Signs – Measurement, Communication & Docum...	—	1 2 3 4	Y / N	
18	Ordering and Explaining Diagnostic Tests	—	1 2 3 4	Y / N	
19	Interpreting and Presenting Lab Results & Imaging	—	1 2 3 4	Y / N	
20	Forming and Communicating a Diagnosis	—	1 2 3 4	Y / N	
21	Treatment Plans – Explaining Options to Patients	—	1 2 3 4	Y / N	
22	Pharmacology English – Prescriptions, Dosages & ...	—	1 2 3 4	Y / N	
23	Explaining Procedures and Obtaining Consent	—	1 2 3 4	Y / N	
24	Post-Treatment Instructions & Follow-Up Care	—	1 2 3 4	Y / N	
25	Emergency Communication – SBAR and Rapid Handover	—	1 2 3 4	Y / N	
26	Writing SOAP Notes in English	—	1 2 3 4	Y / N	
27	Clinical Summaries and Case Reports	—	1 2 3 4	Y / N	
28	Referral Letters and Medical Correspondence	—	1 2 3 4	Y / N	
29	Discharge Summaries	—	1 2 3 4	Y / N	
30	Medical Terminology – Prefixes, Suffixes & Word ...	—	1 2 3 4	Y / N	
31	Communicating with Colleagues – Handovers & Brie...	—	1 2 3 4	Y / N	
32	Case Presentations in English	—	1 2 3 4	Y / N	

Session	Topic	Attendance	Engagement	Objective Met?	Key Observations
33	Multidisciplinary Team (MDT) Meetings	—	1 2 3 4	Y / N	
34	Telehealth & Remote Patient Communication	—	1 2 3 4	Y / N	
35	Reading & Discussing Medical Literature	—	1 2 3 4	Y / N	
36	Breaking Bad News – The SPIKES Protocol	—	1 2 3 4	Y / N	
37	Informed Consent, Patient Rights & Shared Decisi...	—	1 2 3 4	Y / N	
38	Cross-Cultural Communication in Medicine	—	1 2 3 4	Y / N	
39	Ethics, Confidentiality & Professional Boundarie...	—	1 2 3 4	Y / N	
40	Integrated Review, Reflection & Mock Consultatio...	—	1 2 3 4	Y / N	

CHECKLIST 5 – End-of-Course Summative Evaluation

Complete at the end of the 40-session programme to evaluate overall learner progress and course success.

A. Learner Exit Profile – Tick all that apply for each learner

- ☐ Learner can conduct a full patient consultation in English (opening, history, examination, diagnosis, treatment, closing)
- ☐ Learner uses appropriate medical vocabulary with acceptable accuracy
- ☐ Learner can write complete and clear SOAP notes
- ☐ Learner can write a professional referral letter
- ☐ Learner can write a discharge summary
- ☐ Learner gives structured, clear verbal handovers using SBAR or I-PASS
- ☐ Learner delivers a well-organised case presentation
- ☐ Learner participates actively in MDT-style discussions
- ☐ Learner can break bad news using SPIKES framework
- ☐ Learner obtains informed consent using appropriate language
- ☐ Learner adapts language appropriately for different audiences
- ☐ Learner responds empathically to patient distress
- ☐ Learner demonstrates cultural sensitivity in communication
- ☐ Learner can conduct a telehealth consultation
- ☐ Learner reads and discusses medical literature in English

B. Overall Course Success Indicators

- ☐ At least 80% of learners completed the full 40 sessions
- ☐ At least 75% of learners achieved a score of 3 or above on the Oral Communication Checklist
- ☐ At least 75% of learners achieved a score of 3 or above on the Written Communication Checklist
- ☐ All learners demonstrated measurable progress from Session 1 baseline to Session 40
- ☐ Learners reported increased confidence in English clinical communication
- ☐ Teaching quality average score exceeded 3.0 per session
- ☐ Curriculum was delivered in alignment with the published session plan
- ☐ Formative feedback was given at least once per 5 sessions to each learner

C. Final Notes & Recommendations for Future Editions

Sessions requiring additional time:

Topics requiring supplementary materials:

Learner areas of persistent difficulty:

Recommended adaptations for next cohort: